

Universal Catastrophic Coverage with Health Savings Empowerment: A Structural Replacement for the ACA

**A Market-Oriented Framework for Restoring Efficiency
and Fiscal Sustainability in American Health Care**

Technical Policy Brief

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Abstract

Universal Catastrophic Coverage with Health Savings Empowerment (UCC/HSE) proposes a structural replacement for the Affordable Care Act's premium-subsidy architecture. The framework separates American health financing into two distinct layers: universal catastrophic insurance for large, financially destabilizing medical events, and portable Health Savings Empowerment accounts for routine, preventive, pediatric, behavioral-health, and chronic-care needs. The proposal preserves Medicare unchanged and refocuses Medicaid on long-term services and supports, Aged/Blind/Disabled populations, medically complex children, home- and community-based care, and CHIP continuity, while transitioning non-disabled adult acute-care populations into catastrophic coverage and HSE-financed routine care.

The paper argues that the ACA's central fiscal weakness is its reliance on comprehensive insurance subsidies that rise with benchmark premiums. UCC/HSE replaces premium tax credits, cost-sharing reductions, exchange administration, and the tax preference for comprehensive employer-sponsored insurance with bounded catastrophic credits, formula-based HSE deposits, federal high-cost-claim reinsurance, employer-market reform, rural stabilization, and statutory guardrails against benefit creep. Using MEPS HC-251 person-level expenditure data, the model estimates expected excess spending above the deductible threshold rather than treating the deductible itself as federal liability. FY2024 HCRIS provider-impact modeling is used to evaluate routine-care repricing exposure, uncompensated-care relief, bad-debt relief, rural access protection, and transition stabilization.

UCC/HSE is presented as a universal but non-single-payer health-financing architecture. Catastrophic risk is pooled; routine care is financed transparently; vulnerable populations receive targeted liquidity and safeguards; and federal exposure is bounded rather than indexed to comprehensive-plan inflation. The paper concludes that UCC/HSE offers a fiscally disciplined, market-oriented, and institutionally durable alternative to both the ACA's subsidy model and comprehensive federalization of health care.

Table ES-1. UCC/HSE Key Numbers at a Glance

Metric	Estimate
10-year net federal transition cost (Annex E / CBO-style scoring workbook)	+\$763.09B
20-year cumulative federal savings (Annex E / CBO-style scoring workbook)	\$7.462T
30-year cumulative federal savings (Annex E / CBO-style scoring workbook)	\$32.93T
10-year direct consumer savings (Annex E Table 6 / consumer-savings schedule)	\$2.203T
10-year Medicaid consumer savings (Annex E Table 7 / Medicaid transition population)	\$127.92B
10-year federal catastrophic credits (Annex E / HC-251 2023 excess-spending method)	\$6.013T
10-year federal HSA deposits (Annex E / formula-based HSE deposits)	\$5.283T
10-year federal reinsurance (Annex E / 80% above \$50,000 high-cost layer)	\$264.9B
Provider impact before / after stabilization (FY2024 HCRIS provider-impact model)	-\$11.30B → -\$11.08B
Rural impact before / after access protection (FY2024 HCRIS rural / CAH / IHS providers)	-\$0.341B → -\$0.123B

Source note: Federal fiscal estimates are drawn from the UCC/HSE CBO-style scoring workbook and Annex E. Catastrophic-credit estimates use MEPS HC-251 2023 to model expected spending above the deductible rather than treating the deductible itself as the cost unit. Provider-impact estimates are derived from the FY2024 HCRIS provider-impact model.

Note: Annexes are in the [Supplementary Tables](#)

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1. Executive Summary

American health care does not suffer from a shortage of spending. It suffers from a misallocation of spending. The United States spends substantially more on health care than peer high-income countries, yet public programs, employer plans, and households continue to finance a system that routes both catastrophic medical events and routine care through comprehensive insurance. That structure weakens price discipline, inflates administrative costs, obscures routine-care prices, and ties federal subsidy growth to premium inflation rather than to household protection **(1,2,9–14,19,66)**.

The Affordable Care Act expanded coverage, but it did not correct this underlying architecture. Premium tax credits rise with benchmark premiums. Essential health benefit rules preserve comprehensive-plan inflation. Exchange administration adds complexity without restoring price discovery. The result is a system that protects many households from immediate loss of coverage while leaving federal finances exposed to long-term premium growth. CBO's federal health-insurance subsidy projections show the scale of this exposure, including marketplace subsidies, Medicaid and CHIP, Medicare, and employment-based coverage subsidies under current law **(3)**.

Universal Catastrophic Coverage with Health Savings Empowerment offers a structural alternative. The framework separates American health financing into two layers. The first is a universal catastrophic layer that protects eligible individuals against large, financially ruinous medical events. The second is a routine-care financing layer built around portable, prefunded Health Savings Empowerment accounts. Catastrophic risk is pooled and insured. Routine, preventive, pediatric, behavioral-health, and chronic-care needs are financed directly, with targeted support for low-income households, children, and certified chronic-condition patients.

This proposal is not a repeal-only framework, a narrow HSA expansion, or a reduction in catastrophic protection. It is a replacement architecture. UCC/HSE combines standardized catastrophic coverage, federal high-cost-claim reinsurance, formula-based HSE deposits, employer-market reform, Medicaid refocusing, chronic-condition safeguards, rural stabilization, supplemental-coverage guardrails, and statutory fiscal controls. State experience with Section 1332 reinsurance supports the narrower actuarial claim that high-cost-claim stabilization can reduce premiums and stabilize risk pools; it is not used here as stand-alone validation of the full UCC/HSE architecture **(4)**.

Medicare remains unchanged. Traditional Medicare, Medicare Advantage, Part D, Medigap, and Medicare eligibility are outside the UCC/HSE replacement population. Medicaid is not abolished. It is refocused on the populations and services for which Medicaid is

structurally best suited: long-term services and supports, Aged/Blind/Disabled populations, medically complex children, home- and community-based care, and CHIP continuity. Non-disabled adult acute-care populations transition into catastrophic coverage and HSE-financed routine care.

The fiscal rationale is straightforward. The ACA subsidizes comprehensive premiums, and federal obligations rise when those premiums rise. UCC/HSE replaces that open-ended structure with bounded catastrophic credits, fixed HSE formulas, and federal reinsurance. The system is designed to finance catastrophic risk rather than subsidize benefit inflation. Employer-market reform is part of that fiscal structure: the tax exclusion for employer-sponsored insurance encourages compensation to flow into tax-preferred comprehensive health benefits rather than wages or portable health savings, obscuring the wage cost of health benefits and reinforcing premium growth **(21,31–33,64,65)**.

The empirical basis for the model rests on expenditure concentration. MEPS HC-251 person-level expenditure data are used to reconstruct spending across the non-Medicare population. The UCC/HSE model uses this distribution to distinguish routine spending from catastrophic spending and calculates catastrophic liability as expected excess spending above the deductible threshold, rather than treating the deductible itself as federal liability **(46)**. This distinction is central: the deductible is an activation threshold, not the government's cost unit.

The model also recognizes the limits of conventional high-deductible insurance. Evidence on cost-sharing, HSAs, preventive-care use, medication adherence, and chronic-care management shows that poorly designed high-deductible systems can reduce necessary care or fail to generate durable consumer cost-consciousness **(36–40)**. UCC/HSE is designed differently. It pairs catastrophic protection with prefunded routine-care liquidity, pediatric deposits, chronic-condition supplements, and income-sensitive support so that routine-care price discipline is not achieved by simply shifting costs onto low-income or medically vulnerable households.

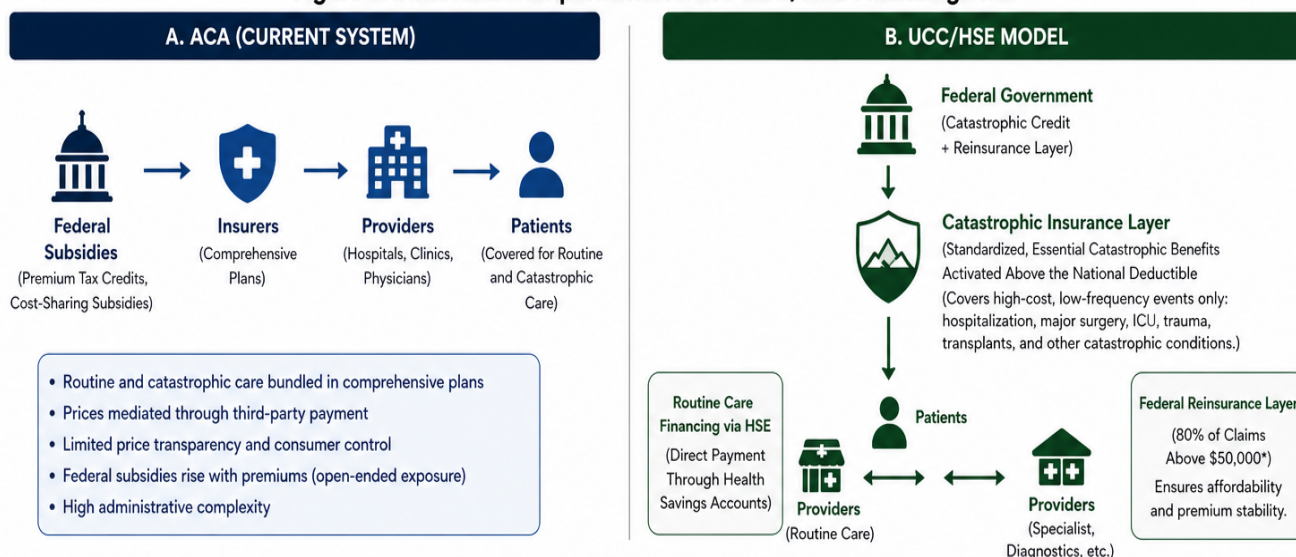
Routine-care price discipline is supported by evidence that prices vary substantially across providers, payers, and sites of service, and that transparency or direct-payment arrangements can reduce payments in selected shoppable service categories **(5–8,20,34,35,53–56)**. UCC/HSE does not rely on consumer shopping alone. It combines routine-care transparency with federal HSE deposits, chronic-condition support, pediatric support, employer-market reform, and a catastrophic backstop.

The model also incorporates provider-impact analysis using FY2024 HCRIS data. Provider effects are modeled as revenue re-composition, not uniform payment cuts. Catastrophic

and high-acuity care remains insured and federally stabilized. Routine and shoppable outpatient care faces greater price discipline. Targeted stabilization is included for rural, pediatric, behavioral-health, and other access-sensitive providers. The provider-impact model is a static, first-order scoring framework; it is not a hospital-closure forecast and does not claim that every provider is held harmless (67).

UCC/HSE therefore seeks to accomplish three goals simultaneously: universal financial protection against catastrophic medical risk, restoration of price discipline in routine care, and a federal fiscal structure that is bounded rather than indexed to comprehensive-plan inflation. It is a universal system without single payer, a market-oriented system without laissez-faire exposure, and a fiscally disciplined system that preserves explicit protections for vulnerable populations.

Figure 1. Structural Comparison: ACA vs UCC/HSE Financing Flow



Note: Diagram simplifies financial flows for conceptual clarity. UCC represents catastrophic-only insurance. HSE represents Health Savings Empowerment accounts used for routine and predictable medical expenditures.

*Policy parameter illustrative; see Annex B.

Table 1. Structural Comparison: ACA vs UCC/HSE

Component	ACA (Current Law)	UCC/HSE Model
Insurance Scope	Comprehensive (routine + catastrophic bundled)	Catastrophic-only coverage
Routine Care Financing	Insurance-mediated, third-party payment	Direct payment via HSA (HSE)
Catastrophic Protection	Included within comprehensive plans	Standardized national catastrophic layer
Federal Subsidies	Premium tax credits (open-ended, indexed to premiums)	Fixed catastrophic credit (bounded exposure)
Price Transparency	Low (prices obscured by insurer-provider contracts)	High (cash-pay and posted pricing)
Administrative Complexity	High (exchanges, AV tiers, EHB mandates, risk adjustment)	Reduced (simplified catastrophic plans + HSA infrastructure)
Employer Role	Central (tax-advantaged ESI system)	Reduced (shift to wages + optional HSA contributions)
Incentive Structure	Encourages benefit expansion and premium growth	Encourages price discipline and cost minimization
Chronic Condition Support	Subsidized via plan design and cost-sharing reductions	Targeted HSA funding + risk-adjusted catastrophic protection
Federal Fiscal Exposure	Expands with healthcare cost growth	Constrained via benchmark-linked catastrophic support
Market Competition	Fragmented, state-based exchange markets	Interstate catastrophic-plan competition
Risk Pooling	Mixed across comprehensive plans	Concentrated in catastrophic layer

Note: HSE (Health Savings Empowerment) refers to portable, tax-advantaged accounts used to finance routine and predictable medical expenditures outside the insurance system.

2. The Structural Problem in U.S. Health Financing

The central problem in American health financing is structural rather than merely fiscal. The United States spends substantially more on health care than peer high-income countries, but the deeper problem is the way that spending is routed. Public programs, employer-sponsored coverage, and ACA exchange products increasingly function at the same time as catastrophic insurance mechanisms, routine-care payment systems, wage-compensation vehicles, and income-transfer structures. This convergence has produced a financing architecture characterized by premium inflation, administrative complexity, opaque routine-care pricing, and rising federal subsidy exposure **(1–3,9–14,19,66)**.

Insurance functions most efficiently when it pools low-frequency, high-severity risk. Catastrophic trauma, major oncology treatment, organ failure, intensive-care hospitalization, severe neurological disease, and other financially destabilizing medical events fit this framework because they are uncertain, concentrated, and economically ruinous for households. Routine primary care, maintenance medications, predictable specialist visits, standard imaging, low-acuity outpatient services, behavioral-health follow-up, and recurring chronic-disease management have different economic characteristics. They may be clinically important and financially burdensome, but they are generally more predictable and more responsive to direct liquidity support, transparent pricing, and targeted subsidies than catastrophic hospitalization or high-severity acute care.

This distinction is central to UCC/HSE. The claim is not that below-threshold care is trivial. For low-income households, children, and chronic-condition patients, predictable routine spending can still be burdensome. The relevant question is whether those needs are best financed through comprehensive premium pooling or through direct liquidity support beneath a catastrophic backstop. UCC/HSE answers that question by separating the financing mechanism from the access-protection mechanism: catastrophic risk is pooled through insurance, while routine care is financed through prefunded Health Savings Empowerment accounts, targeted deposits, and transparent pricing.

The distinction is also visible in the expenditure distribution itself. Health spending is highly skewed: most individuals incur modest annual expenses, while a smaller share accounts for a disproportionate share of total spending. A financing system that treats both categories as equivalent obscures the economic boundary between routine liquidity and catastrophic risk pooling. **Section 4** reconstructs this distribution using MEPS HC-251 person-level expenditure data and shows why the catastrophic threshold is modeled as expected excess spending above the deductible rather than as the deductible itself **(46)**.

When routine expenditures are financed through comprehensive third-party insurance, prices become insulated from direct consumer visibility. Providers increasingly compete through network inclusion, negotiated reimbursement, and billing leverage rather than transparent prices. Consumers are rarely exposed to the true marginal cost of routine services, while insurers and provider systems devote increasing resources to utilization management, coding systems, network construction, reimbursement negotiation, prior authorization, and administrative adjudication. Comprehensive insurance therefore expands far beyond catastrophic protection and becomes the primary payment intermediary for ordinary medical consumption.

The result is not merely higher spending, but distorted spending. International comparisons show that the United States spends substantially more per capita and as a share of GDP than other developed economies. A major component of that divergence is administrative complexity. The U.S. system devotes substantial resources to billing, insurance administration, benefit determination, claims processing, regulatory compliance, and provider-insurer contracting. Administrative growth is not an accidental byproduct of American health care; it is a predictable consequence of routing routine medical transactions through complex insurer-provider adjudication structures **(2,9–14,18,19)**.

The Affordable Care Act stabilized portions of the individual insurance market through premium tax credits, cost-sharing reductions, actuarial value requirements, guaranteed issue protections, essential health benefit rules, and exchange-based enrollment systems. Those reforms expanded coverage and reduced some forms of medical financial exposure. However, the ACA largely preserved the underlying comprehensive-insurance framework. Federal support remains linked to benchmark premiums, meaning that federal obligations rise automatically when comprehensive-plan premiums rise. CBO's federal subsidy projections show the scale of this exposure under current law **(3)**.

This relationship creates an inherently inflation-sensitive fiscal structure. Under the ACA model, the federal government does not merely subsidize catastrophic protection; it subsidizes comprehensive premium growth itself. As benchmark premiums increase due to provider consolidation, benefit expansion, administrative complexity, utilization growth, or pricing power, federal subsidies rise in parallel. The system therefore ties long-run federal obligations to the inflation trajectory of comprehensive insurance products rather than to a measurable catastrophic risk layer.

The employer-sponsored insurance market reinforces these dynamics. The tax exclusion for employer-sponsored health insurance remains one of the largest federal tax expenditures. Because employer-paid premiums are excluded from taxable income,

employers and employees have strong incentives to channel compensation into comprehensive insurance benefits rather than taxable wages or portable health savings. Economically, the exclusion subsidizes comprehensive-plan expansion and contributes to the persistence of routine-care financing through third-party insurance **(21,31–33,64,65)**.

This structure has several downstream consequences. First, compensation increasingly migrates toward premium benefits rather than direct wages. Second, routine-care pricing becomes progressively insulated from competitive pressure because consumers interact with negotiated copays, coinsurance structures, and network restrictions rather than transparent market prices. Third, insurers compete heavily through benefit design, network engineering, and reimbursement contracting rather than catastrophic-risk pooling alone. Finally, federal expenditures become increasingly exposed to premium growth across both employer and exchange markets. Rising employer premiums also reduce the visibility of total compensation and make health costs appear institutionally separate from wages, even though workers ultimately bear much of the cost through lower cash compensation **(31–33,64,65)**.

Price opacity compounds these structural problems. Routine outpatient imaging, laboratory testing, specialist visits, ambulatory procedures, and elective services frequently exhibit substantial price variation across providers, payers, and sites of service. Evidence from commercial-price studies, FAIR Health consumer-price data, hospital-price-transparency research, and direct-payment studies shows that prices for shoppable and routine services can vary widely and that transparency or direct-payment arrangements can reduce payments in selected categories **(5–8,16,17,20,34,35,53–56)**. Yet consumers often lack meaningful price information before services are delivered, and insurers frequently negotiate reimbursement arrangements that obscure underlying transaction costs. In many routine-care settings, the financing system suppresses ordinary market signals that would otherwise discipline pricing behavior.

Provider consolidation intensifies these dynamics. Large hospital systems, vertically integrated networks, and consolidated insurer-provider arrangements can use market power to negotiate higher commercial reimbursement rates. In comprehensive insurance markets, insurers often must maintain broad networks to remain competitive, while consolidated providers negotiate from increasingly powerful positions. CBO and empirical hospital-price research both show that commercial prices can substantially exceed Medicare prices and vary widely across markets **(6,8,34)**. The result is an environment in which negotiated reimbursement rates can rise without corresponding improvements in price transparency, routine-care competition, or clinical value.

These dynamics do not imply that health care functions as a perfectly competitive commodity market. Nor do they imply that catastrophic protection should be weakened. Catastrophic medical events remain uniquely suited to pooled insurance because their financial risk is unpredictable, concentrated, and potentially ruinous. The problem arises when the same insurance architecture used for catastrophic trauma and oncology treatment is also used to finance ordinary, recurring, and partially predictable routine consumption.

Nor does the argument imply that simple high-deductible insurance is sufficient. Evidence on cost-sharing, HSAs, preventive-care use, medication adherence, and chronic-care management shows that poorly designed high-deductible arrangements can reduce necessary care or fail to produce durable consumer cost-consciousness **(36–40)**. UCC/HSE is therefore not a conventional high-deductible plan. It pairs catastrophic protection with prefunded routine-care liquidity, pediatric deposits, chronic-condition supplements, and income-sensitive support so that routine-care price discipline is not achieved by merely shifting predictable costs onto low-income or medically vulnerable households.

The cumulative result is a system in which households face rising premiums, employers face rising compensation costs, providers operate within increasingly complex reimbursement systems, and federal obligations continue expanding alongside benchmark premium inflation. UCC/HSE is built around the premise that these structural problems cannot be solved solely through additional subsidies, mandates, or comprehensive-plan regulation. A different financing architecture is required.

The UCC/HSE framework therefore separates catastrophic risk from routine consumption. Catastrophic events remain universally insured through pooled protection and federal high-cost-claim stabilization. Routine expenditures increasingly transition toward directly financed care supported by portable Health Savings Empowerment accounts, targeted federal liquidity support, and transparent pricing structures beneath the catastrophic threshold. The objective is not to eliminate public protection or shift catastrophic liability onto households. The objective is to restore a distinction between insurance and routine payment that the current system increasingly lacks.

3. Core UCC/HSE Architecture

Universal Catastrophic Coverage with Health Savings Empowerment restructures American health financing around a division between catastrophic insurance and routine-care financing. The model preserves universal protection against financially ruinous medical events while replacing the ACA's subsidy-driven comprehensive-insurance architecture with a catastrophic-first framework supported by portable Health Savings Empowerment accounts, federal high-cost-claim reinsurance, employer-market re-composition, and statutory fiscal guardrails.

The replacement population is the non-Medicare population. Medicare remains outside the reform. Traditional Medicare, Medicare Advantage, Medigap, Part D, and Medicare eligibility are not altered. Medicaid is not abolished; it is refocused on long-term services and supports, Aged/Blind/Disabled populations, medically complex children, home- and community-based care, and CHIP continuity. UCC/HSE applies primarily to the non-Medicare acute-care financing architecture that is currently fragmented across ACA exchange coverage, Medicaid expansion and other non-disabled adult Medicaid pathways, employer-sponsored insurance, and uninsured household exposure.

The framework is designed around five principles. First, catastrophic medical risk should remain universally insured. Second, routine and partially predictable expenditures should increasingly operate within transparent and directly financed markets. Third, federal obligations should be bounded rather than mechanically linked to comprehensive premium inflation. Fourth, vulnerable populations should receive targeted support through explicit financing mechanisms rather than through broad premium subsidization. Fifth, institutional safeguards must prevent the gradual reconstruction of inflationary comprehensive-plan dynamics beneath the catastrophic threshold.

The structural contrast is summarized in **Figure 1 and Table 1**. Under the ACA, routine and catastrophic expenditures are bundled inside comprehensive insurance products whose premiums are supported by federal subsidies and employer tax preferences. Under UCC/HSE, catastrophic risk is pooled through a standardized catastrophic layer, while routine and predictable care is financed through HSE accounts, direct payment, employer contributions, and targeted federal liquidity support. This distinction is the central design choice: UCC/HSE does not eliminate health coverage, but changes what is insured, what is directly financed, and what federal subsidies are designed to support.

UCC/HSE is not a copy of any foreign health system, but its financing logic has international analogues. Singapore separates catastrophic protection from medical savings through MediShield Life and Medisave; the Netherlands demonstrates that

standardized national coverage can coexist with regulated insurer competition; and Switzerland preserves a stronger deductible-based distinction between routine spending and insured risk than the United States. These systems differ substantially from UCC/HSE, but they support the narrower proposition that catastrophic protection, household-level routine-care financing, and regulated insurance competition can coexist inside universal health systems **(23–27)**.

Universal Catastrophic Coverage

The catastrophic layer forms the foundation of the UCC/HSE system. Under the baseline structure, eligible individuals receive standardized catastrophic coverage protecting against major medical expenditures exceeding a defined deductible threshold. Catastrophic insurance therefore functions as true insurance: protection against severe financial loss resulting from unpredictable, high-cost medical events.

The deductible is an activation threshold, not the federal cost unit. UCC/HSE estimates catastrophic exposure using expected excess spending above the deductible threshold rather than assuming that the deductible itself represents the government’s liability. This distinction is central to the fiscal structure of the proposal. The catastrophic layer prices the insured portion of medical spending while preserving direct financing responsibility for routine expenditures occurring below the catastrophic trigger. The MEPS HC-251 reconstruction described in Section 4 provides the empirical basis for this expected-excess method **(46)**.

The catastrophic benchmark is derived from expected excess spending above the threshold, not from comprehensive ACA premiums. This matters because comprehensive premiums bundle routine benefits, administrative load, provider pricing power, mandated benefits, network design, and catastrophic risk into a single price. UCC/HSE separates those components. The insured layer is priced according to catastrophic exposure, while the routine layer is financed through HSE accounts and direct payment.

Catastrophic plans remain federally standardized to ensure universal access to major medical protection. High-acuity hospitalization, trauma care, major surgery, intensive-care services, severe acute illness, transplant-related hospitalization, and other catastrophic events remain insured and federally stabilized. The objective is not reduced catastrophic protection, but narrower and more economically coherent use of insurance itself.

The model also incorporates federal catastrophic credits tied to benchmark catastrophic-plan pricing rather than comprehensive-plan inflation. Because catastrophic coverage insures a narrower risk layer than ACA-style comprehensive plans, federal exposure becomes more bounded and less sensitive to routine-care inflation occurring beneath the

deductible threshold. This structure contrasts with the current ACA subsidy architecture, in which federal support remains tied to benchmark premiums **(3)**.

Health Savings Empowerment Accounts

Routine expenditures are financed through Health Savings Empowerment accounts. Unlike traditional Health Savings Accounts, HSE accounts are not merely tax-advantaged savings vehicles operating at the margins of the insurance system. They are integrated into the national financing architecture and serve as the primary financing mechanism for routine and predictable medical expenditures beneath the catastrophic threshold.

HSE accounts combine household contributions, employer contributions, and formula-based federal deposits. Federal support varies according to income, pediatric status, age, and certified chronic-condition status. The accounts are portable across employment transitions and are individually controlled rather than tied permanently to employer-sponsored comprehensive plans.

Routine primary care, preventive services, outpatient medications, standard laboratory testing, low-acuity specialist visits, behavioral-health services, and many recurring chronic-condition expenditures are financed through the HSE layer. The model therefore seeks to create greater routine-care price visibility while preserving targeted support for households with elevated medical needs.

The framework explicitly rejects the assumption that all households possess identical routine-care capacity. Low-income households, children, and individuals with certified chronic conditions receive enhanced liquidity support through formula-based federal deposits, pediatric allocations, early-year HSE deposits, and chronic-condition supplements. These mechanisms reduce underutilization risk and maintain continuity of essential care access beneath the deductible threshold.

The objective is not pure consumer exposure or uncompensated cost shifting. Conventional high-deductible designs can reduce necessary care, weaken preventive-service use, or fail to create durable consumer cost-consciousness if households lack liquidity or chronic-care support. UCC/HSE therefore pairs direct routine-care financing with prefunded HSE deposits and targeted chronic-condition support rather than relying on deductible exposure alone **(36–40)**.

Household-Facing Structure

For households, the practical structure is intentionally simpler than the ACA's architecture. A household has catastrophic protection for major medical events, an HSE account for routine and predictable care, and targeted federal deposits where income, pediatric status,

or chronic-condition certification warrants support. Routine services are paid through the HSE account, employer contributions, household deposits, direct primary care arrangements, or transparent cash-pay markets. High-cost episodes crossing the catastrophic threshold are handled through the catastrophic plan.

This creates a clearer division of responsibility. Routine care is financed through a portable account with visible prices. Major medical losses are insured. Low-income households and medically vulnerable populations receive additional financing beneath the deductible rather than being placed into separate comprehensive insurance tiers. The household therefore does not lose catastrophic protection; it experiences a different financing structure for routine care.

Federal High-Cost-Claim Reinsurance

Federal high-cost-claim reinsurance stabilizes the catastrophic insurance layer by absorbing a defined portion of extremely high-cost claims above specified attachment points. State reinsurance experience under Section 1332 waivers demonstrates that high-cost-claim stabilization can reduce premium volatility and support individual-market stability (4). UCC/HSE generalizes this logic within a standardized catastrophic framework.

The rationale for reinsurance is grounded in expenditure concentration. A relatively small share of patients accounts for a disproportionate share of aggregate spending. Federal reinsurance therefore focuses stabilization resources on the upper tail of the expenditure distribution rather than subsidizing routine spending broadly across the entire population.

This approach differs structurally from ACA-style premium subsidies. Rather than subsidizing increasingly comprehensive plans across all spending categories, reinsurance targets concentrate catastrophic exposure directly. The federal government assumes a bounded role focused on severe financial risk rather than comprehensive routine-care financing.

Employer-Market Re-composition

UCC/HSE does not abolish employer participation in health financing. Instead, it restructures the employer market gradually over time. Under the current system, employers channel large portions of compensation toward tax-advantaged comprehensive insurance premiums. Under UCC/HSE, compensation increasingly migrates toward wages, employer HSE contributions, portable catastrophic-plan support, and limited supplemental benefits.

This transition reduces the distortionary effects of the employer tax exclusion while preserving employer participation in worker health financing. Employers may continue

contributing toward employee coverage and routine-care funding, but the structure becomes increasingly portable and individually controlled rather than dependent on employer-managed comprehensive plans. The employer tax exclusion remains one of the largest federal tax expenditures, and prior economic work links rising employer health premiums to labor-market and wage effects **(21,31–33,64,65)**.

The proposal does not require abrupt elimination of employer-sponsored insurance. Transition mechanisms allow gradual migration from comprehensive employer plans toward catastrophic-first financing structures. Collective-bargaining continuity, staged tax-exclusion phase-down, and employer HSE contribution pathways reduce disruption while shifting federal tax policy away from subsidizing comprehensive premium growth.

Chronic-Condition and Pediatric Safeguards

A catastrophic-first model requires explicit safeguards for households with predictable medical needs beneath the deductible. UCC/HSE therefore includes chronic-condition and pediatric financing mechanisms within the HSE layer rather than creating separate insurance tiers.

Certified chronic-condition patients receive enhanced HSE deposits, condition-based support, and routine-care liquidity for predictable disease-management needs. Pediatric routine care is financed through age-adjusted HSE deposits and coordinated with Medicaid and CHIP continuity rules. These mechanisms preserve a uniform catastrophic insurance product while recognizing that predictable routine-care needs differ by clinical status, age, and household income.

This structure prevents a common failure mode in high-deductible designs. UCC/HSE does not simply expose households to a deductible and assume that all routine-care needs can be privately absorbed. It pairs catastrophic protection with prefunded routine-care liquidity for the groups most likely to face access risk. The design responds directly to evidence that cost-sharing and high-deductible arrangements can reduce preventive-care use, medication adherence, or guideline-concordant chronic care when liquidity and chronic-condition supports are insufficient **(36–40)**.

Supplemental-Coverage Guardrails

One of the major institutional risks facing catastrophic-first systems is the gradual reconstitution of comprehensive insurance through supplemental products. Without statutory guardrails, insurers and employers would have strong incentives to recreate low-deductible comprehensive-plan structures beneath the catastrophic threshold, eventually reproducing the same inflationary dynamics present under the ACA framework.

UCC/HSE therefore incorporates supplemental-coverage restrictions designed to preserve the distinction between catastrophic insurance and routine-care financing. Supplemental products remain permissible, but the system restricts their ability to function as de facto comprehensive first-dollar insurance at scale. Supplemental products may address narrowly defined ancillary or elective needs, but they may not functionally rebuild ACA-style comprehensive insurance with federal support.

These guardrails are institutionally important because the long-run fiscal logic of the proposal depends on maintaining separation between catastrophic pooled risk and directly financed routine expenditures. Without such protections, comprehensive-plan inflation would likely reemerge through supplemental layering and employer-benefit competition.

What Changes and What Does Not Change

UCC/HSE changes the financing architecture of the non-Medicare acute-care market, but it does not eliminate universal catastrophic protection or abolish existing public programs.

Medicare remains structurally unchanged. Traditional Medicare, Medicare Advantage, Medigap, and Part D continue operating outside the replacement population. Medicaid remains focused on long-term services and supports, Aged/Blind/Disabled populations, medically complex children, home- and community-based care, and CHIP continuity. Catastrophic hospitalization, trauma care, major oncology episodes, emergency surgery, and other high-acuity medical events remain insured.

The principal structural change occurs beneath the catastrophic threshold. Routine-care financing increasingly transitions away from comprehensive premium structures and toward portable HSE-based financing combined with targeted federal support and catastrophic backstop protection.

For most households, the practical experience of the system changes in several ways simultaneously. Catastrophic financial protection remains intact. Routine expenditures become more transparent and directly financed. Employers continue participating in health financing but through more portable contribution structures. Federal support becomes more explicitly tied to catastrophic protection and targeted liquidity assistance rather than comprehensive premium inflation.

The resulting system is intended to function as a universal but non-single-payer architecture: catastrophic risk remains pooled and insured, routine care becomes increasingly transparent and directly financed, vulnerable populations receive targeted support, and federal fiscal exposure becomes more bounded over the long term.

4. Empirical Basis: MEPS Catastrophic-Spending Reconstruction

The UCC/HSE framework rests on a basic empirical fact: health-care spending is highly concentrated. Most individuals incur modest annual medical costs, while a smaller share of the population accounts for a disproportionate share of aggregate expenditures. That distributional pattern is central to the design of catastrophic insurance. It suggests that health financing should distinguish between ordinary, recurring, and relatively low-variance medical consumption on the one hand, and high-cost, high-variance medical events on the other.

This distinction is not merely conceptual. It can be measured directly using person-level expenditure data. The UCC/HSE model uses the Medical Expenditure Panel Survey Household Component Full-Year Consolidated Data File, HC-251, to reconstruct the distribution of annual health-care expenditures across the non-Medicare population. MEPS HC-251 provides nationally representative person-level data on total annual medical expenditures, demographic characteristics, insurance status, age, and survey weights. Those weights allow individual survey observations to be scaled to national population totals **(46)**.

The purpose of this reconstruction is not to estimate average medical spending. Average spending alone is insufficient for catastrophic-plan design. A catastrophic insurance model depends on the distribution of spending around a threshold: how many individuals exceed a defined spending level, how much spending occurs above that level, how much remains below it, and whether above-threshold spending is concentrated enough to justify separate risk pooling. The UCC/HSE model therefore focuses on expenditure concentration and expected excess spending above the catastrophic threshold.

4.1 Why expenditure concentration matters

A system that treats all medical spending as economically equivalent will tend to over-insure routine services and under-discipline prices. Routine care and catastrophic care have different economic properties. Routine primary care, standard laboratory testing, low-acuity imaging, generic medications, behavioral-health follow-up, and stable chronic-condition monitoring are recurring and often predictable. These services require financing and liquidity support, but they do not ordinarily require pooled insurance in the same way as trauma, intensive-care admission, major cancer treatment, transplant-related hospitalization, or other severe medical events.

Catastrophic events are different. They are high-cost, difficult to predict at the individual level, and capable of imposing financial ruin. These are the events for which insurance is

most economically appropriate. The central task is therefore to identify the point in the spending distribution where routine financing gives way to catastrophic risk.

MEPS expenditure concentration provides the empirical basis for that boundary. In the reconstructed non-Medicare population, a minority of individuals accounts for a majority of spending above the modeled catastrophic threshold. This pattern supports the core UCC/HSE architecture: routine and predictable care is financed through Health Savings Empowerment accounts, while high-cost spending above the threshold is pooled through universal catastrophic coverage (46).

Figure 2 illustrates the concentration pattern underlying UCC/HSE. The curve orders individuals by annual medical spending and compares cumulative population share with cumulative spending share. A large share of the population accounts for relatively little spending, while the upper portion of the distribution accounts for most expenditures. This is the empirical foundation for distinguishing between routine-care financing and catastrophic risk pooling.

4.2 MEPS HC-251 and the non-Medicare reconstruction

The model uses MEPS HC-251 because it allows health spending to be reconstructed at the person level. Each observation includes total annual medical expenditures and a population weight. The model applies those weights to estimate national spending totals and population counts.

The relevant population for UCC/HSE is the non-Medicare replacement population. This is a necessary design constraint. UCC/HSE does not replace Medicare. Traditional Medicare, Medicare Advantage, Part D, Medigap, and Medicare eligibility remain outside the reform. Therefore, Medicare beneficiaries must be excluded from the catastrophic-credit and HSE replacement population.

This exclusion is not a cosmetic adjustment. Medicare beneficiaries account for a substantial share of national health spending, especially high-cost spending. Including them in the UCC/HSE expenditure base while also preserving Medicare would double-count a large federal coverage population and overstate the cost of the UCC/HSE replacement system. The model therefore filters MEPS observations by Medicare status and constructs the catastrophic-spending distribution for the non-Medicare population.

The resulting non-Medicare distribution becomes the empirical basis for estimating UCC/HSE catastrophic exposure. It is this population, not the entire U.S. population and not the Medicare population, that is used to calculate the share of individuals exceeding the catastrophic threshold and the spending above that threshold.

Figure 2. MEPS-Based Concentration Curve of Health-Care Expenditures, Non-Medicare Population

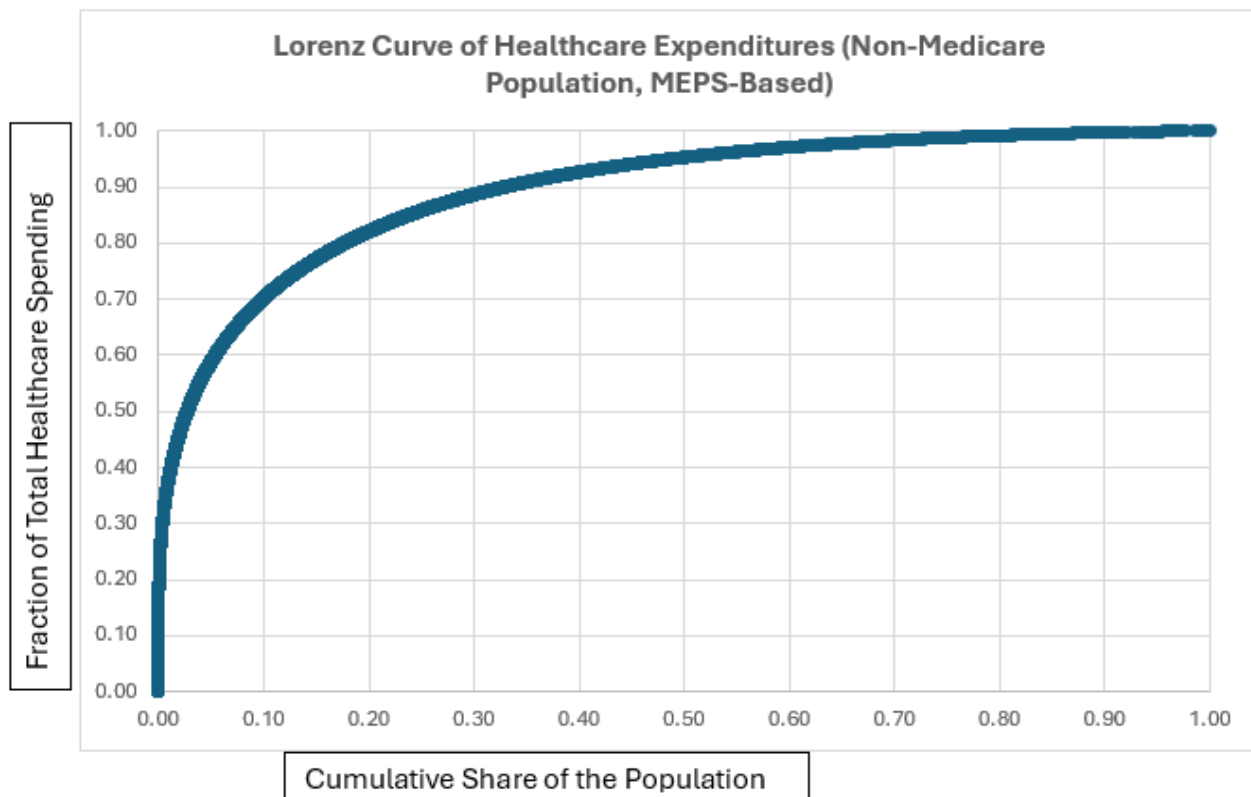


Figure 2: This figure displays the cumulative distribution of healthcare spending across the non-Medicare population using MEPS HC-251 data. Individuals are weighted to reflect national totals and ordered by annual expenditures. The curve illustrates the extreme concentration of healthcare costs, where a minority of individuals account for most of the total spending.

Source: Author reconstruction using MEPS HC-251 person-level expenditure data and survey weights. (46)

4.3 The \$7,500 threshold

The baseline model uses a \$7,500 individual catastrophic threshold, with a broader statutory range of approximately \$7,500 to \$10,000 for adults and \$15,000 to \$20,000 for couples or family-equivalent units. The \$7,500 value is not chosen as a symbolic high deductible. It is used because the MEPS reconstruction indicates that this range captures an important transition point in the expenditure distribution **(46)**.

At the \$7,500 threshold, the model estimates that approximately 40.73 million non-Medicare persons exceed the threshold in annual medical spending. This represents approximately 15.10 percent of the weighted non-Medicare population. The spending above that threshold is approximately \$778.91 billion, representing approximately 55.08 percent of total reconstructed non-Medicare medical spending. Below-threshold spending is approximately \$635.14 billion, or 44.92 percent of total reconstructed non-Medicare spending **(46)**.

This result is important. It means that a minority of the non-Medicare population accounts for a majority of spending above the threshold. The distribution is therefore consistent with the use of a catastrophic risk pool. At the same time, the below-threshold spending share remains large enough that routine-care financing must be designed carefully. UCC/HSE does not ignore this lower-spending portion of the distribution. It finances routine care through HSE accounts, income-based early-year deposits, pediatric funding, chronic-condition support, and targeted liquidity mechanisms.

The threshold therefore performs two roles. Actuarially, it defines the activation point for catastrophic coverage. Institutionally, it preserves the distinction between pooled insurance and directly financed routine care.

4.4 Expected excess spending above the threshold

The most important scoring principle is that the deductible is not the government's cost unit. A deductible is an activation threshold. It determines where catastrophic coverage begins. It does not by itself determine the cost of the catastrophic layer.

The relevant actuarial quantity is expected excess spending above the threshold. For each individual i , let: X_i

represent total annual medical spending for individual i , and let: d

represent the catastrophic deductible threshold. Individual excess spending above the threshold is:

$$(X_i - d)^+ = \max(0, X_i - d)$$

Individuals whose spending falls below the threshold contribute zero to catastrophic liability. Individuals whose spending exceeds the threshold contribute only the amount above the threshold.

In weighted MEPS form, total above-threshold spending for the eligible non-Medicare population is calculated as:

$$\text{Total Excess}_d = \sum_i w_i \cdot M_i \cdot \max(0, X_i - d)$$

where: w_i

is the MEPS population weight, and: M_i is an indicator equal to 1 if the individual is in the eligible non-Medicare population and 0 otherwise.

The corresponding per-person expected excess exposure is:

$$\text{Expected Excess Per Eligible Person}_d = \frac{\sum_i w_i \cdot M_i \cdot \max(0, X_i - d)}{\sum_i w_i \cdot M_i}$$

This is the central catastrophic benchmark input. It measures expected above-threshold spending per eligible non-Medicare person, not the deductible and not the average spending of all people who trigger coverage.

The same quantity can also be expressed as:

$$p_d \times E[(X_i - d) \mid X_i \geq d, M_i = 1]$$

Where:

$$p_d = \frac{\sum_i w_i \cdot M_i \cdot \mathbf{1}(X_i \geq d)}{\sum_i w_i \cdot M_i}$$

is the weighted share of eligible non-Medicare persons whose annual spending exceeds the threshold. This identity clarifies the model logic: catastrophic liability depends on both the trigger rate and the amount of spending above the threshold among those who trigger.

This method is essential because it preserves correct threshold sensitivity. If the deductible is lowered, expected catastrophic exposure rises because more individuals cross the threshold and more dollars fall above it. If the deductible is raised, expected exposure falls because fewer individuals cross the threshold and fewer dollars fall above it. A model that multiplies the deductible by the number of enrollees, or by the share of enrollees exceeding the threshold, would not measure catastrophic liability correctly.

For example, if an individual spends \$5,000 and the threshold is \$7,500, that person contributes zero to catastrophic liability. If an individual spends \$20,000, that person contributes \$12,500 in spending above the threshold before applying federal credit shares, insurer liability, household premium shares, or reinsurance rules. The deductible defines where the insured layer begins; it is not the amount paid by the federal government.

This distinction also prevents a mechanical error in benchmark-premium modeling. If the deductible itself is treated as the cost unit, a lower deductible can appear to reduce modeled liability simply because the threshold value has fallen. That is backwards. In a correct catastrophic model, lowering the threshold increases liability because more spending becomes insured. The expected-excess method avoids this error.

4.5 Simplified MEPS threshold calculations

Table 2 summarizes the empirical breakpoint used in the model. Approximately 15 percent of the non-Medicare population exceeds \$7,500 in annual spending, yet spending above that threshold represents roughly 55 percent of total reconstructed non-Medicare spending. Among adults, the triggered population is approximately 34.99 million, or 17.87 percent of adult non-Medicare persons. Average total spending among non-Medicare persons above the threshold is approximately \$19,123, and average total spending among adult non-Medicare persons above the threshold is approximately \$22,262 **(46)**.

These figures are consistent with a system in which most individuals experience routine or moderate annual spending, while a smaller group experiences high-cost medical needs that account for a disproportionate share of total expenditures. The spending distribution therefore supports a financing architecture that uses HSE accounts for routine care and universal catastrophic coverage for the upper tail of spending.

4.6 From MEPS reconstruction to catastrophic benchmark input

The MEPS reconstruction does not by itself produce the federal outlay. It produces the empirical spending distribution used to derive the catastrophic benchmark. The scoring model then applies policy parameters to that benchmark, including the federal catastrophic credit schedule, household premium contribution assumptions, insurer liability, federal reinsurance parameters, and growth assumptions.

The key bridge from MEPS to the scoring workbook is expected excess spending per eligible enrollee. The workbook first calculates total weighted excess spending above the threshold. It then divides that amount by the weighted eligible non-Medicare population. This produces an expected per-person above-threshold exposure. That value is then scaled to the UCC/HSE-covered population and used to estimate the catastrophic premium or benchmark exposure associated with the standardized catastrophic plan.

This procedure is more defensible than importing comprehensive-plan premiums directly into the UCC/HSE model. Comprehensive ACA premiums reflect a mixture of catastrophic risk, routine-care benefits, administrative overhead, benefit mandates, network pricing, and insurer pricing behavior. UCC/HSE is designed to insure a narrower layer of risk. Its catastrophic benchmark should therefore be derived from the distribution of spending above the deductible, not from the premium structure of comprehensive insurance products.

Table 2. Simplified MEPS HC-251 Catastrophic-Threshold Calculations: Non-Medicare Spending Above \$7,500

<i>Metric</i>	MEPS HC-251 weighted estimate
<i>Total non-Medicare medical spending</i>	\$1.414 trillion
<i>Weighted non-Medicare population</i>	~266 million
<i>Non-Medicare persons with annual spending $\geq$ \$7,500</i>	40.73 million
<i>Share of non-Medicare population with spending $\geq$ \$7,500</i>	15.10%
<i>Adults 18+ and non-Medicare with spending $\geq$ \$7,500</i>	34.99 million
<i>Share of adult non-Medicare population with spending $\geq$ \$7,500</i>	17.87%
<i>Weighted spending above \$7,500 threshold</i>	\$778.91 billion
<i>Share of total non-Medicare spending above threshold</i>	55.08%
<i>Weighted spending below \$7,500 threshold</i>	\$635.14 billion
<i>Share of total non-Medicare spending below threshold</i>	44.92%
<i>Average total spending among non-Medicare persons above threshold</i>	\$19,123
<i>Average total spending among adult non-Medicare persons above threshold</i>	\$22,262

Source: Author reconstruction using MEPS HC-251 person-level expenditure data, survey weights, total annual expenditure variables, age variables, and Medicare-status filtering.
(46)

4.7 Alignment with national spending projections

MEPS provides the distributional shape of health spending, but CMS National Health Expenditure projections and CMS National Health Expenditure highlights provide the macro-spending context for forward projection **(1,66)**. The UCC/HSE scoring framework uses MEPS to reconstruct the spending distribution around the catastrophic threshold, then aligns that distribution with national health-spending trends for longer-horizon scoring.

This distinction matters. MEPS is a survey-based microdata source and is not a full CBO microsimulation. It is used here to reconstruct the distribution of person-level medical expenditures and to estimate expected excess spending above a defined threshold. CMS projections provide the national spending-growth context. The UCC/HSE model then applies its own policy assumptions regarding catastrophic credits, HSE deposits, reinsurance, administrative simplification, routine-care repricing, and employer-market re-composition.

The model therefore separates three analytic layers:

1. Distributional reconstruction: MEPS HC-251 provides the person-level spending distribution and threshold exposure **(46)**.
2. Macro alignment: CMS projections provide national health-spending growth context **(1,66)**.
3. Policy scoring: UCC/HSE parameters translate the reconstructed exposure into federal outlays and savings.

This separation improves transparency. The data source does not imply the policy result. The policy result emerges from explicit modeling assumptions applied to an observed expenditure distribution.

4.8 Relationship to the routine-care layer

The same distributional logic that supports catastrophic pooling also supports HSE-financed routine care. Below-threshold spending is not ignored. It represents a substantial share of total non-Medicare spending and includes services that households use regularly: primary care, behavioral-health follow-up, standard laboratory testing, low-acuity imaging, routine specialist visits, generic medications, preventive services, and chronic-condition monitoring.

Under the current comprehensive-insurance architecture, these services are routed through premium financing, insurer billing, negotiated reimbursement, coding systems,

and administrative adjudication. Under UCC/HSE, they are financed through HSE accounts, direct payment, transparent cash prices, and targeted federal liquidity support. The purpose is to expose routine-care prices to greater discipline while ensuring that low-income households, children, and chronic-condition patients have sufficient prefunded resources to maintain access.

The MEPS threshold does not imply that all below-threshold spending is optional or trivial. It indicates that below-threshold spending is economically different from catastrophic spending. Routine-care financing requires liquidity, price transparency, and targeted support. Catastrophic spending requires risk pooling. UCC/HSE separates these functions rather than forcing both through comprehensive insurance.

4.9 Treatment of children, adults, and income bands

The threshold table also shows why the model must distinguish between population categories. All-person counts, adult counts, non-Medicare counts, and adult non-Medicare counts are not interchangeable. The all-person triggered count at \$7,500 is approximately 70.60 million. The adult triggered count is approximately 63.58 million. The non-Medicare triggered count is approximately 40.73 million. The adult non-Medicare triggered count is approximately 34.99 million **(46)**.

For UCC/HSE scoring, the non-Medicare filter is central because Medicare remains outside the replacement system. Adult non-Medicare values are useful for adult deductible calibration, while all non-Medicare values are relevant for total covered-population exposure. Pediatric financing is handled separately through age-adjusted HSE deposits and preserved Medicaid/CHIP protections for specified child populations. This prevents the catastrophic threshold from being misapplied to children's predictable routine-care needs.

The adult income-band distribution in the master calculation further shows that threshold exposure varies across income groups, but not in a way that justifies separate catastrophic insurance products. UCC/HSE therefore preserves a uniform catastrophic plan while adjusting financing beneath the deductible through HSE deposits, income-based support, chronic-condition supplements, and pediatric funding.

4.10 Limitations of the MEPS reconstruction

The MEPS-based reconstruction is a transparent distributional estimate. It is not a substitute for a full CBO, CMS, MedPAC, or actuarial microsimulation. Several limitations should be acknowledged.

First, MEPS is survey-based and may understate aggregate national spending relative to CMS National Health Expenditure totals. For that reason, the model uses MEPS primarily to

estimate the distributional shape of expenditures and aligns forward projections with CMS spending trends **(1,44,46,66)**.

Second, MEPS measures annual expenditures but does not fully capture all insurer contracting behavior, provider bargaining dynamics, network effects, or long-run equilibrium responses under a new national catastrophic architecture. Those dynamics require future actuarial review.

Third, the reconstruction estimates spending above a threshold using total annual expenditures. It does not, by itself, classify every claim into a statutory UCC-covered or excluded category. The statutory benefit design, high-cost drug carve-outs, reinsurance rules, and benchmark-linked payment limits must be applied in the scoring framework after the distributional baseline is constructed.

Fourth, the model is based on a national distribution and does not fully capture regional price variation, local provider concentration, or state-specific utilization patterns. Those factors are relevant for implementation, reinsurance calibration, and provider-impact analysis.

These limitations do not undermine the use of MEPS for the technical policy paper. They clarify its role. MEPS provides a reproducible empirical foundation for estimating where catastrophic risk lies in the expenditure distribution. It does not eliminate the need for subsequent actuarial scoring.

4.11 Implications for UCC/HSE design

The MEPS reconstruction strengthens the UCC/HSE framework in four ways.

First, it grounds the catastrophic threshold in observed expenditure distribution rather than political preference. The \$7,500 threshold identifies a point at which a minority of non-Medicare individuals accounts for a majority of above-threshold spending.

Second, it keeps Medicare outside the replacement population, preserving consistency between the empirical model and the policy design.

Third, it prices catastrophic liability as expected excess spending above the threshold, not as the deductible itself. This prevents the model from understating liability when the threshold falls or overstating simplicity by assuming every enrollee reaches the deductible.

Fourth, it clarifies why routine care and catastrophic care require different financing mechanisms. The upper tail of spending is suitable for pooled insurance. Below-threshold care requires a different architecture: HSE deposits, transparent prices, routine-care liquidity, chronic-condition support, pediatric funding, and income-based safeguards.

The empirical reconstruction therefore serves as the quantitative bridge between the paper's diagnosis and its proposed architecture. The current system routes routine and catastrophic care through the same comprehensive insurance structure. The MEPS distribution shows why that structure is economically imprecise. UCC/HSE responds by financing routine care directly while insuring catastrophic risk through a bounded, measurable, and nationally pooled layer.

5. Federal Fiscal Scoring Framework

The fiscal logic of UCC/HSE differs from the fiscal logic of the ACA. The ACA subsidizes comprehensive insurance premiums, and federal exposure rises as benchmark premiums rise. UCC/HSE instead finances a defined catastrophic layer, deposits routine-care support into Health Savings Empowerment accounts, and stabilizes high-cost claims through federal reinsurance. Federal obligations are therefore shifted away from open-ended comprehensive-plan inflation and toward defined, formula-based categories that can be scored, indexed, and constrained.

The estimates in this section are internal CBO-style model estimates, not official Congressional Budget Office scores. The scoring framework uses current-law federal subsidy baselines, MEPS HC-251 catastrophic-spending reconstruction, CMS national spending projections, employer-exclusion tax expenditure estimates, and the UCC/HSE workbook's outlay and savings schedules. The purpose is not to replace a formal CBO score, but to present a transparent fiscal architecture that can be reviewed, recalibrated, and stress-tested under standard budget conventions **(1,3,21,44,46,66)**.

The scoring framework has three basic components. First, the model estimates gross federal outlays under UCC/HSE: catastrophic credits, federal HSE deposits, reinsurance, and transition or administrative costs. Second, it estimates offsetting savings and revenues relative to current law: ACA subsidy replacement, Medicaid adult acute-care transition, employer-exclusion reform, administrative simplification, routine-care repricing, and related structural savings. Third, it compares gross outlays with offsetting savings to estimate net federal impact over 10-, 20-, and 30-year horizons.

5.1 Federal Outlay Categories

Federal outlays under UCC/HSE are grouped into three primary categories: catastrophic credits, federal HSE deposits, and federal reinsurance. These are the core fiscal commitments created by the reform.

Catastrophic Credits

Catastrophic credits finance a defined share of the benchmark catastrophic premium for eligible households. These credits replace ACA-style premium tax credits tied to comprehensive benchmark plans. The fiscal distinction is important. Under the ACA, the federal subsidy is attached to a premium that includes routine benefits, mandated coverage categories, provider-network pricing, administrative load, and catastrophic risk. Under UCC/HSE, the federal credit is attached to a narrower catastrophic product.

The catastrophic-credit calculation is based on the expected cost of the insured catastrophic layer, not on comprehensive-plan premiums. As shown in Section 4, the MEPS reconstruction estimates expected excess spending above the catastrophic threshold. That expected-excess value informs the benchmark catastrophic premium, to which federal credit parameters are then applied. The credit is therefore connected to a measurable catastrophic risk layer rather than to the full cost of comprehensive first-dollar coverage **(46)**.

The model applies income-sensitive credit parameters to avoid indiscriminate subsidy expansion. Catastrophic protection remains universal, but the federal share of premium support is calibrated through statutory formulas and fiscal guardrails. This structure preserves universal financial protection while reducing the open-ended character of federal subsidy exposure.

Federal HSE Deposits

Federal HSE deposits finance routine-care liquidity beneath the catastrophic threshold. These deposits include baseline adult deposits, pediatric deposits, chronic-condition supplements, income-indexed support, and recession-triggered additions where applicable. The deposits are treated as mandatory outlays because they represent direct federal transfers into eligible HSE accounts.

The HSE-deposit category is essential to the design. UCC/HSE is not a pure high-deductible model that exposes households to routine costs without assistance. The system removes routine care from comprehensive insurance but replaces that financing channel with explicit routine-care liquidity. Low-income households, children, and chronic-condition patients receive additional support because predictable below-threshold spending can still impose real financial burdens. This design responds to evidence that conventional high-deductible arrangements can reduce preventive-care use, weaken medication adherence, or fail to generate durable consumer cost-consciousness when liquidity and chronic-condition support are inadequate **(36–40)**.

The model distinguishes between fixed chronic-condition supplements and income-sensitive baseline deposits. Chronic-condition supplements are attached to certified clinical need. Income sensitivity applies to the baseline deposit component, not to the chronic supplement itself. This preserves the distinction between poverty-related liquidity constraints and diagnosis-related routine-care burden.

Federal Reinsurance

Federal reinsurance stabilizes the catastrophic insurance layer by absorbing a defined share of claims above high-cost attachment points. This reduces insurer exposure to extreme claims and lowers volatility in catastrophic-plan pricing. State reinsurance programs under Section 1332 waivers provide evidence that high-cost-claim stabilization can reduce premiums and stabilize risk pools, although those state experiences are narrower than the full UCC/HSE architecture (4).

Reinsurance is not the entire reform. It is the tail-risk stabilizer. UCC/HSE's long-run fiscal impact depends on the combination of reinsurance, catastrophic credits, HSE deposits, employer-exclusion reform, routine-care repricing, administrative simplification, and fiscal guardrails. Reinsurance makes catastrophic premiums more predictable; it does not by itself generate the 20- and 30-year savings estimates.

5.2 Federal Savings and Revenue Categories

The savings side of the model is scored relative to current law. UCC/HSE replaces or reduces federal spending and tax expenditure categories associated with comprehensive insurance. It also incorporates structural savings from administrative simplification, routine-care repricing, employer-market re-composition, and provider-sector transition. The major savings categories are summarized below.

ACA Subsidy Replacement

UCC/HSE eliminates the ACA's premium-linked subsidy architecture for the replacement population. Premium tax credits, cost-sharing reductions, exchange administration, actuarial-value compliance, and related subsidy mechanisms are replaced by catastrophic credits and HSE deposits. The fiscal effect is not simply the removal of ACA outlays; it is the substitution of one financing architecture for another. ACA subsidies are tied to comprehensive benchmark premiums. UCC/HSE outlays are tied to catastrophic exposure and formula-based routine-care deposits.

CBO's health-insurance subsidy baselines provide the current-law reference point for marketplace subsidies, Medicaid/CHIP, employment-based tax subsidies, and other

federal health-insurance subsidy categories. The UCC/HSE scoring workbook uses those categories to define the baseline against which replacement savings are measured **(3)**.

Employer-Exclusion Reform

The employer tax exclusion is one of the largest federal health-related tax expenditures. UCC/HSE phases down the federal tax preference for comprehensive employer-sponsored insurance and redirects employer participation toward HSE deposits, catastrophic-plan facilitation, wages, and limited supplemental benefits **(21,31–33,64,65)**.

This reform generates federal revenue effects because compensation that was previously excluded from taxable income is gradually converted into taxable wages or non-excluded compensation. The model does not assume immediate or complete wage pass-through. It applies a phased reduction schedule and adjusts for partial pass-through and behavioral friction. This prevents the model from overstating revenue gains while still recognizing that employer-paid premiums are part of total labor compensation.

In the current workbook, the employer-exclusion baseline is modeled at approximately \$451.0 billion in 2026, \$578.0 billion in 2030, and \$759.6 billion in 2035, with a 2026–2035 total of approximately \$5.998 trillion. The corresponding phased revenue gain is approximately \$45.1 billion in 2026, \$289.0 billion in 2030, and \$759.6 billion in 2035, totaling approximately \$3.569 trillion over ten years.

Administrative Simplification

The ACA and employer-sponsored comprehensive insurance markets generate substantial administrative complexity: exchange operation, eligibility determination, subsidy reconciliation, risk adjustment, actuarial-value compliance, network contracting, prior authorization, benefit design, billing, claims adjudication, and insurer-provider contracting. UCC/HSE reduces administrative burden by standardizing catastrophic coverage, removing routine care from comprehensive claims administration, and replacing exchange-based subsidy machinery with simpler federal credits and HSE deposits. Evidence on U.S. administrative cost burdens, billing-related costs, Medicare Advantage administration, and cross-national billing and insurance-related administrative costs supports the premise that administrative complexity is a major contributor to U.S. health spending **(10–14,18,19)**.

The model does not assume that administrative costs disappear. Catastrophic claims still require adjudication. HSE accounts require custodial administration. Reinsurance requires auditing and claims verification. Medicaid LTSS/ABD functions remain administratively complex. Program integrity systems remain necessary. Administrative savings are therefore applied only to the categories plausibly affected by UCC/HSE, not to total national health expenditure indiscriminately.

Routine-Care Repricing

Routine-care repricing is a structural savings mechanism, not a simple across-the-board utilization cut. When routine services are financed through HSE accounts, direct payment, transparent cash prices, and direct primary care arrangements, routine prices face greater pressure than under opaque comprehensive insurance. The model applies routine-care savings only to the spending categories plausibly affected by HSE-financed direct payment: routine primary care, standard imaging, laboratory testing, low-acuity outpatient services, and other below-threshold services.

This category is deliberately separated from catastrophic spending. The model does not assume that patients shop for trauma surgery, ICU admission, or emergency oncology care. Those services remain insured through the catastrophic layer. Routine-care repricing applies to the portion of spending where transparent prices and consumer-directed payment are institutionally plausible. The evidence base for this mechanism includes RAND price-transparency and direct-payment work, CBO analysis of commercial prices, FAIR Health consumer-price data, hospital price-transparency rules, and empirical research on shoppable-service price variation **(5–8,16,17,20,34,35,53–56)**.

Other Structural Savings

The workbook also includes secondary savings mechanisms such as uncompensated-care reduction, bad-debt reduction, avoided emergency-department overutilization, reduced diagnostic duplication, site-of-service migration, and provider-sector re-composition. These categories are treated conservatively because they are more sensitive to behavioral assumptions than direct subsidy replacement or tax-expenditure reform. Some are modeled as central-case savings; others are treated as limited or sensitivity-only effects. Emergency-department and uncompensated-care sources support the direction of these mechanisms, while provider-sector estimates depend on the FY2024 HCRIS-based provider-impact model **(47–52,67)**.

This distinction is important for credibility. The model’s primary fiscal engine is not speculative behavioral savings. It is the structural replacement of premium-linked subsidies with bounded catastrophic credits, fixed HSE formulas, employer-exclusion reform, and benchmark-linked catastrophic pricing. Behavioral and utilization effects strengthen the fiscal trajectory but do not carry the full model.

5.3 Compressed Federal Scoring Summary

Table 3 summarizes the current modeled federal impact. The estimates are internal CBO-style model outputs, not official CBO scores.

Table 3. Compressed Annex E Fiscal Summary: UCC/HSE Net Federal Impact

<i>Fiscal measure</i>	Modeled result
<i>Gross federal outlays, 2026–2035</i>	\$11.570 trillion
<i>Net federal impact, 2026–2035</i>	+\$763.09 billion
<i>Cumulative federal impact, 20-year horizon</i>	-\$7.462 trillion
<i>Cumulative federal impact, 30-year horizon</i>	-\$32.93 trillion
<i>Direct consumer savings, 2026–2035</i>	\$2.203 trillion
<i>Medicaid transition-population consumer savings, 2026–2035</i>	\$127.92 billion

Note: Positive net federal impact indicates a transition cost relative to the ACA/current-law baseline. Negative cumulative impact indicates federal savings relative to baseline. These are internal CBO-style estimates, not official CBO scores.

The central fiscal result is transitional. UCC/HSE costs more than the current ACA subsidy baseline over the first ten years because the reform simultaneously finances catastrophic protection and prefunds routine-care HSE deposits. This is the front-loaded cost of replacing an existing premium-subsidy architecture with a new catastrophic-and-HSE structure.

The model turns positive over longer horizons because the old system and the new system grow at different rates. Under the ACA, federal obligations remain tied to comprehensive benchmark premiums and employer tax preferences. Under UCC/HSE, federal obligations are tied to a defined catastrophic layer, fixed deposit formulas, reinsurance parameters, and statutory fiscal guardrails. Over time, that difference compounds.

5.4 Merged Outlays and Savings Components

Table 4 summarizes the principal outlay and savings components in a condensed form. The outlay totals are reported directly from the current model. The savings/revenue total is the aggregate offset required to reconcile gross outlays with the reported 10-year net federal impact.

Table 4. Merged Federal Outlays and Savings Components, 2026–2035

Component	Budget treatment	10-year modeled amount
<i>Catastrophic credits</i>	Federal outlay	\$6.013 trillion
<i>Federal HSE deposits</i>	Federal outlay	\$5.283 trillion
<i>Federal reinsurance</i>	Federal outlay	\$264.9 billion
<i>Total gross UCC/HSE federal outlays</i>	Federal outlay subtotal	\$11.570 trillion
<i>ACA subsidy replacement, Medicaid adult acute-care transition, employer-exclusion reform, administrative simplification, routine-care repricing, and other modeled savings/revenues</i>	Aggregate savings/revenue effects	~\$10.807 trillion
<i>Identified employer-exclusion phase-down revenue gain</i>	Revenue effect within aggregate savings	\$3.569 trillion
<i>Net federal impact, 2026–2035</i>	Outlays minus savings/revenues	+\$763.09 billion

Note: The aggregate savings/revenue line includes ACA subsidy replacement, employer-exclusion reform, Medicaid adult acute-care transition, administrative simplification, routine-care repricing, and other modeled savings categories. The employer-exclusion amount is shown separately because it is a major identifiable revenue component within the broader savings architecture.

This table clarifies the fiscal structure. UCC/HSE is not cheap in gross outlay terms. It creates large federal commitments through catastrophic credits and HSE deposits. Its claim to fiscal sustainability rests on whether the replacement architecture displaces larger or faster-growing current-law spending over time. The model’s 10-year transition cost reflects that the reform front-loads routine-care deposits and catastrophic protection before long-run structural savings fully compound.

5.5 Why Savings Compound Over Time

The long-run fiscal improvement arises from compounding differences between the ACA/current-law structure and the UCC/HSE structure.

First, ACA premium tax credits rise with benchmark premiums. If benchmark premiums reflect comprehensive benefit inflation, provider pricing power, administrative load, or insurer-market dynamics, federal subsidies rise as well. UCC/HSE breaks that link by tying federal catastrophic support to a defined catastrophic product rather than to comprehensive premium growth **(3)**.

Second, HSE deposits are formula-based rather than premium-indexed. Federal routine-care support is explicit, bounded, and adjustable through statutory formulas. It does not rise automatically because a comprehensive plan's actuarial value, mandated benefits, or provider-network prices rise.

Third, employer-exclusion reform compounds over time. As the tax preference for comprehensive employer-sponsored insurance phases down, federal revenues rise and compensation becomes more transparent. The transition is gradual, but the budget effect grows as the exclusion is reduced and as employer compensation shifts toward wages, HSE deposits, or narrower supplemental benefits **(21,31–33,64,65)**.

Fourth, administrative simplification grows as legacy ACA exchange machinery, comprehensive routine-care claims processing, and employer-plan complexity shrink. These savings do not appear instantly; they accumulate as households, employers, insurers, and providers migrate to the new architecture **(10–14,18,19)**.

Fifth, routine-care repricing is cumulative. HSE-financed direct payment does not instantly transform every market. But as routine care moves away from comprehensive insurance and toward transparent pricing, providers face stronger incentives to post prices, compete on routine services, and reduce billing overhead. The model treats these changes as gradual structural effects rather than immediate one-year cuts **(5–8,16,17,20,34,35,53–56)**.

Sixth, provider-sector re-composition changes the long-run spending path. UCC/HSE preserves insured payment for catastrophic and high-acuity care, but routine outpatient services become more exposed to price discipline. Over time, revenue migrates away from opaque facility-fee billing and toward independent clinics, direct primary care, ambulatory surgery centers, imaging centers, laboratories, and efficient specialist practices. This is not scored as a simple provider cut; it is modeled as a change in the financing channel and site-of-care economics **(53–56,67)**.

5.6 Fiscal Guardrails

The scoring framework depends on maintaining guardrails. Without them, UCC/HSE could reproduce the same inflationary dynamics it is designed to replace. The principal guardrails are:

1. Catastrophic credits are tied to a defined catastrophic benchmark rather than comprehensive premiums;
2. Fixed and formula-based HSE deposits;
3. Reinsurance parameters with defined attachment points;
4. Employer-exclusion phase-down rules;
5. Restrictions on federally subsidized supplemental coverage;
6. Deductible and deposit indexation limits;
7. Annual fiscal monitoring and automatic parameter adjustment if outlays exceed statutory caps.

These guardrails distinguish UCC/HSE from a broad new entitlement. The framework creates universal catastrophic protection, but it also defines the federal obligation in statutory and actuarial terms. Its fiscal credibility depends on preserving those boundaries.

5.7 Interpretation of the 10-Year Transition Cost

The 10-year transition cost should not be interpreted as evidence that the framework fails fiscally. It reflects the timing of reform. UCC/HSE begins by financing two forms of protection simultaneously: catastrophic protection for high-cost events and HSE deposits for routine-care liquidity. The savings from employer-market re-composition, administrative simplification, routine-care repricing, and provider-sector changes emerge more gradually.

The relevant fiscal question is therefore not whether the first ten years show zero transition cost. The relevant question is whether the model replaces a faster-growing subsidy architecture with a slower-growing, bounded architecture. Under the current model, the answer is yes: the first decade produces a transition cost, while the second and third decades produce large cumulative savings relative to baseline.

This is the central budgetary claim of UCC/HSE. It is not that the system avoids federal spending. It is that federal spending is redirected toward catastrophic protection and routine-care liquidity in a structure less exposed to comprehensive-plan inflation.

6. Medicaid, Medicare, and Vulnerable Populations

The strongest objection to catastrophic-first reform is that high deductibles can harm low-income and medically vulnerable populations. That objection is valid when catastrophic designs are built around exposure alone. UCC/HSE is structured differently. The model separates the insurance product from the financing beneath the deductible. The catastrophic plan remains uniform, but routine-care liquidity varies according to income, age, pediatric status, and certified chronic-condition need. Evidence on high-deductible plans, HSAs, preventive-service use, medication adherence, and chronic-care management supports the concern that poorly designed cost-sharing can reduce necessary care if liquidity and clinical safeguards are absent **(36–40)**.

This distinction is central. UCC/HSE does not protect vulnerable populations by recreating ACA-style comprehensive insurance tiers. It protects them through targeted financing beneath a universal catastrophic backstop. The objective is to preserve a simple catastrophic insurance structure while ensuring that households with predictable routine-care needs are not forced into medical debt, deferred care, or avoidable emergency-department use.

6.1 Medicare Remains Unchanged

Medicare remains outside the UCC/HSE replacement population. Traditional Medicare, Medicare Advantage, Part D, Medigap, and Medicare eligibility are not altered. Older Americans continue to receive benefits under existing Medicare law. The UCC/HSE model therefore does not use Medicare savings to finance the reform and does not rely on Medicare benefit reductions as a fiscal mechanism.

This separation is necessary for both policy and scoring reasons. Medicare beneficiaries represent a high-cost and older population with a distinct statutory financing structure. Including Medicare beneficiaries in UCC/HSE would distort the catastrophic-spending reconstruction and create unnecessary transition risk. UCC/HSE instead focuses on the non-Medicare acute-care financing architecture: ACA marketplaces, Medicaid adult acute-care coverage, employer-sponsored comprehensive plans, and uninsured household exposure.

Medicare noninterference also strengthens the political and administrative feasibility of the model. A reform that reopened Medicare benefits, Medicare Advantage contracts, Medigap rules, or Part D would become a fundamentally different project. UCC/HSE is designed as a structural replacement for non-Medicare health financing, not as a Medicare reform.

6.2 Medicaid Is Refocused, Not Abolished

Medicaid remains essential for populations whose needs do not fit ordinary acute-care insurance. UCC/HSE preserves Medicaid for long-term services and supports, Aged/Blind/Disabled populations, medically complex children, home- and community-based care, and CHIP continuity. These populations and services are structurally different from the non-disabled adult acute-care population. They involve disability, long-term care, functional support, pediatric complexity, or continuous care coordination rather than standard acute medical risk.

The model therefore transitions non-disabled adult acute-care populations into UCC/HSE while preserving Medicaid's core institutional role. Non-disabled adults receive catastrophic protection through UCC and routine-care liquidity through HSE accounts. Medicaid continues to finance the services for which it is uniquely suited: LTSS, HCBS, ABD coverage, medically complex children, and state-administered safety-net functions that cannot be replaced by a standard catastrophic product.

This design avoids two failures. It avoids abolishing Medicaid functions that are essential for disabled and medically complex populations. It also avoids using Medicaid as a broad acute-care financing vehicle for populations who can instead be covered through catastrophic protection and HSE-financed routine care.

6.3 Non-Disabled Adult Acute-Care Transition

The primary Medicaid transition population consists of non-disabled adults whose current coverage is oriented toward acute medical care rather than LTSS or disability-related services. Under UCC/HSE, these individuals move into the same catastrophic insurance structure as other eligible non-Medicare adults. Routine and predictable care is financed through HSE deposits, income-based support, and chronic-condition supplements where applicable.

This transition is not a withdrawal of catastrophic protection. It changes the financing channel. Hospitalization, trauma, severe acute illness, emergency surgery, and other high-cost events remain insured through the catastrophic layer. Routine care is financed through prefunded accounts and transparent purchasing mechanisms.

The model also estimates consumer savings for the affected Medicaid transition population. In the current scoring workbook, Medicaid transition-population consumer savings are estimated at approximately \$127.92 billion over 2026–2035. These savings arise from a combination of lower routine-care prices, reduced administrative friction, HSE-funded routine-care access, and continued protection against catastrophic medical expenses.

6.4 Chronic-Condition HSE Support

Chronic-condition patients require special treatment within a catastrophic-first system. Many chronic conditions generate predictable, recurring expenses that may not always cross the catastrophic threshold but can still be financially burdensome. A poorly designed high-deductible model would expose these patients to routine-care underuse and preventable deterioration. UCC/HSE addresses this problem through chronic-condition HSE support beneath the deductible **(36–40)**.

Certified chronic-condition patients receive additional HSE deposits and may participate in condition-based routine-care bundles. The catastrophic insurance product remains the same for everyone. What varies is the financing beneath the deductible. This preserves the simplicity of a uniform catastrophic plan while addressing predictable disease-management costs.

The chronic support mechanism is designed for conditions such as diabetes, congestive heart failure, COPD, chronic kidney disease monitoring, stable behavioral-health needs, addiction-management follow-up, and other conditions requiring recurring outpatient care. Covered routine needs may include standard labs, monitoring visits, medication management, generic medications, care-plan review, and routine specialist follow-up. High-severity complications remain in the catastrophic layer.

This structure avoids creating a separate chronic-condition insurance tier. It uses targeted HSE financing and risk-adjusted support to protect patients without undermining the uniform catastrophic architecture.

6.5 Pediatric Routine-Care Funding

Children require a separate routine-care financing logic because pediatric preventive services are predictable, clinically important, and relatively low variance. Well-child visits, immunizations, developmental screening, routine hearing and vision screening, low-acuity pediatric visits, and standard pediatric labs do not fit the economic definition of catastrophic risk. They are better financed through prefunded pediatric HSE deposits coordinated with Medicaid and CHIP continuity rules. MEPS pediatric expenditure sources and child-health preventive-care files support the need to treat pediatric routine care separately from adult catastrophic-threshold calibration **(42,43,45)**.

UCC/HSE therefore includes age-adjusted pediatric routine-care deposits. These deposits preserve access to essential pediatric care without routing predictable pediatric services through comprehensive insurance. High-severity pediatric events, trauma, oncology,

congenital anomalies, severe infections, and medically complex pediatric conditions remain protected through catastrophic coverage, Medicaid medically complex child protections, or preserved CHIP/Medicaid continuity where applicable.

The pediatric structure is important for political and substantive reasons. A catastrophic-first system cannot appear indifferent to children's routine care. UCC/HSE avoids that risk by treating pediatric routine care as predictable, essential, and prefunded beneath the catastrophic layer.

6.6 Income-Based Early-Year HSE Support

A common failure of high-deductible designs is the early-year liquidity problem. Households may face medical bills before they have accumulated sufficient savings. Low-income households are most vulnerable to this problem. Without prefunding, they may defer routine care, substitute emergency departments for primary care, or accumulate medical debt. Evidence on high-deductible arrangements and HSAs supports the concern that account-based designs fail when liquidity is inadequate or when patients with chronic needs face early-year exposure without sufficient support **(36–40)**.

UCC/HSE addresses this through income-based early-year HSE support. Federal HSE deposits are made at the beginning of the benefit year for qualifying low-income households and chronic-condition patients. This makes routine-care liquidity available before bills arise. The deposit is not a retrospective reimbursement and not a claim-by-claim insurance benefit. It is prefunded purchasing power for routine care.

This mechanism is central to the model's equity claim. UCC/HSE does not ask low-income households to wait for market forces to reduce prices before accessing care. It gives them routine-care funds up front while preserving catastrophic protection for high-cost events.

6.7 ER Non-Backflow Guardrail

The ER non-backflow guardrail addresses a predictable objection: if routine care is removed from comprehensive insurance, low-income patients may defer care and return to emergency departments. UCC/HSE is designed to prevent that outcome through early-year HSE deposits, chronic-condition support, pediatric funding, and routine-care liquidity.

The goal is to prevent predictable outpatient needs from becoming avoidable emergency visits. Routine primary care, medication management, chronic monitoring, behavioral-health follow-up, and low-acuity services should be financed before they deteriorate into emergency utilization. AHRQ/HCUP emergency-department cost estimates and health-system analyses of avoidable emergency-department use support the concern that failed outpatient access can shift costs into emergency settings **(47,48)**.

The guardrail does not recreate comprehensive first-dollar insurance. It preserves the catastrophic/routine distinction while ensuring that the routine-care layer has enough liquidity to function. Low-income households receive funds early enough to purchase routine services, maintain medications, and manage chronic needs without relying on emergency departments as the default access point.

6.8 No Separate Insurance Tier for Vulnerable Populations

UCC/HSE protects vulnerable populations through financing adjustments rather than insurance-tier fragmentation. The catastrophic plan remains uniform. Deductible structure, covered catastrophic services, and reinsurance parameters remain standardized. What changes is the amount and timing of HSE support beneath the deductible.

This design has three advantages. First, it avoids creating high-risk insurance tiers that could become stigmatized or underfunded. Second, it preserves actuarial simplicity in the catastrophic layer. Third, it targets federal resources toward actual routine-care burden rather than subsidizing comprehensive-plan inflation for all households regardless of need.

The result is progressive support inside a uniform catastrophic architecture. Low-income households receive prefunded liquidity. Children receive pediatric routine-care support. Chronic-condition patients receive diagnosis-based routine-care financing. Medically complex children and ABD populations remain protected through Medicaid. Medicare beneficiaries remain outside the reform. CHIP continuity is preserved.

6.9 Continuity for Children and Transitional Medicaid Populations

Coverage continuity is essential during implementation. Children enrolled in Medicaid or CHIP should not experience coverage interruption during the transition to UCC/HSE. The model therefore preserves Medicaid/CHIP continuity while coordinating pediatric HSE deposits and catastrophic enrollment for children who transition into the UCC/HSE structure.

For transitional Medicaid populations, including households with fluctuating income, postpartum extensions, or redetermination timing issues, UCC/HSE uses automatic enrollment and coordinated eligibility processes to prevent gaps in catastrophic protection. The transition is designed to avoid duplicative enrollment while ensuring that no eligible household loses catastrophic coverage because of administrative timing.

This continuity framework is particularly important for children and medically vulnerable households. The reform's credibility depends on demonstrating that vulnerable populations are not treated as an afterthought. UCC/HSE therefore combines automatic

catastrophic protection, prefunded routine-care liquidity, preserved Medicaid functions, and CHIP continuity.

6.10 Answering the “Who Gets Hurt?” Objection

The vulnerable-population critique is the most important test of any catastrophic-first proposal. If a model protects healthy, high-income households while exposing low-income and medically complex populations, it fails as a national reform. UCC/HSE is designed to avoid that failure.

Medicare beneficiaries are not disrupted. Medicaid LTSS, ABD populations, medically complex children, HCBS, and CHIP continuity are preserved. Non-disabled adult acute-care populations receive catastrophic protection and HSE-financed routine care. Chronic-condition patients receive enhanced support beneath the deductible. Children receive pediatric routine-care financing. Low-income households receive early-year HSE deposits. The ER non-backflow guardrail prevents routine-care liquidity gaps from becoming avoidable emergency utilization.

The central policy judgment is that vulnerable populations require targeted financing, not a return to comprehensive-plan inflation. UCC/HSE protects vulnerable households by making routine-care funds available where needed while preserving universal catastrophic insurance for high-cost events. That approach maintains access without rebuilding the ACA’s open-ended premium-subsidy structure.

7. Provider Impact and Rural Stabilization

A structural replacement for the ACA must answer a practical question: what happens to hospitals, physicians, rural providers, and other delivery-system institutions when routine care moves away from comprehensive insurance and into transparent HSE-financed markets? The UCC/HSE provider-impact model is designed to answer that question without reducing the issue to a simple provider-payment cut. The provider-sector effect of UCC/HSE is best understood as revenue re-composition with targeted access stabilization, not as a uniform reduction in reimbursement.

The framework does not impose a Medicare-for-All-style conversion of all commercial payments to public payment rates. It does not rely on a single administered reimbursement schedule for every hospital and physician service. Its effect arises from the separation of financing channels. Catastrophic and high-acuity care remains insured, pooled, and federally stabilized. Routine, shoppable, and lower-acuity outpatient care shifts toward HSE-financed direct payment and transparent pricing. The resulting provider impact is therefore a reallocation of revenue pressure away from opaque, high-markup routine billing and toward a system in which high-severity care remains insured, routine care becomes

more price-disciplined, and access-sensitive providers receive targeted stabilization **(5–8,20,34,35,53–56)**.

This distinction is central. UCC/HSE preserves insured payment for inpatient hospitalization, trauma care, ICU services, major surgery, transplant-related hospitalization, severe acute illness, high-acuity oncology episodes, and other catastrophic events. These services remain within the catastrophic insurance layer and are supported by catastrophic credits and federal high-cost-claim reinsurance. The model does not assume that patients shop for emergency surgery, trauma stabilization, or intensive-care admission. Those are precisely the kinds of events insurance is meant to cover.

The principal provider-sector adjustment occurs beneath the catastrophic threshold. Routine office visits, standard imaging, laboratory testing, chronic-care monitoring, low-complexity outpatient procedures, routine specialist follow-up, and other predictable services currently routed through comprehensive insurance become more exposed to transparent pricing. In markets where hospital outpatient departments rely on facility-fee billing, negotiated commercial rates, or opaque bundled charges for routine services, UCC/HSE applies pressure. In markets where independent clinics, ambulatory surgery centers, imaging centers, laboratories, direct primary care practices, and efficient specialist groups can compete on cash prices, UCC/HSE creates opportunity **(5–8,20,34,35,53–56)**.

7.1 Provider Impact as Revenue Re-composition

Provider impact under UCC/HSE is not measured by applying a blanket percentage reduction to all hospital revenue. That approach would misstate the reform. The model separates high-acuity insured care from routine outpatient exposure. Catastrophic revenue remains largely protected through the insured layer. Routine outpatient exposure is modeled separately because that is where HSE-financed price discipline is expected to operate.

This approach also avoids overstating the provider effect. A hospital's total revenue is not equivalent to its repricing exposure. A trauma center, children's hospital, rural critical access hospital, or academic medical center may have substantial inpatient and high-acuity revenue that remains inside the catastrophic layer. By contrast, a hospital outpatient department with large volumes of shoppable imaging, routine diagnostics, or low-complexity ambulatory procedures may face greater repricing pressure. The model therefore evaluates exposure by service type, institutional role, and provider class.

UCC/HSE's provider-sector logic can be summarized in three propositions. First, catastrophic care remains insured. Second, routine outpatient exposure is repriced. Third, access-sensitive providers receive bounded stabilization where routine-care price discipline could otherwise threaten essential capacity. This is the provider-side analogue of the household-side architecture: risk is insured, routine care is priced more transparently, and vulnerable access points receive targeted support.

7.2 HCRIS Provider-Impact Model

The provider-impact model uses FY2024 CMS Healthcare Cost Report Information System hospital cost-report data to estimate static, first-order provider revenue effects under UCC/HSE. The model uses a provider backbone of 912 records, of which 886 have usable financial and exposure data for scoring. Twenty-six records are excluded because required financial or exposure variables are missing, unusable, or fail validation (67).

The exposure base is net outpatient exposure. This is derived from gross outpatient revenue adjusted by the provider's net-to-gross ratio, defined as net patient revenue divided by gross total patient revenue. This construction estimates the portion of outpatient and routine revenue plausibly exposed to UCC/HSE price discipline. It does not treat total hospital revenue as exposed. It also does not assume that inpatient catastrophic care, trauma care, ICU services, transplant-related admissions, or major high-acuity episodes migrate into cash-priced routine markets.

The model also corrects provider classification before scoring. Raw HCRIS labels are not sufficient for policy analysis because administrative categories may fail to distinguish large academic systems, children's hospitals, rural critical access providers, behavioral-health institutions, rehabilitation facilities, and specialty hospitals. The scoring model therefore uses cleaned analytic provider classes rather than raw administrative labels alone. The principal modeled groups include general acute/academic providers, children's/pediatric providers, rural/CAH/IHS providers, and specialty/behavioral/rehabilitation providers.

The baseline provider-impact equation is:

$$\text{Net Provider Impact} = -\text{Market Repricing Pressure} + \text{UC/Liquidity Offset} + \text{Bounded Stabilization Support}$$

Market repricing pressure captures routine/outpatient exposure subject to HSE price discipline. The UC/liquidity offset captures relief from uncompensated care and liquidity-related access failures. Bounded stabilization support captures targeted implementation-period protection for providers facing acute adjustment pressure. The model therefore recognizes both the negative revenue effect of routine-care repricing and the positive

provider-facing effects of lower uncompensated care, better household liquidity, and transition support (49–52,67).

Table 7. Stabilized Provider-Impact Model: Base/Base and Preferred Access Protection

<i>Provider-impact measure</i>	Base/Base model	Preferred access-protection sensitivity
<i>Data source</i>	FY2024 CMS HCRIS hospital cost reports	FY2024 CMS HCRIS hospital cost reports
<i>Provider backbone</i>	912 providers	912 providers
<i>Modeled providers</i>	886 providers	886 providers
<i>Excluded providers</i>	26 providers	26 providers
<i>Total net patient revenue</i>	~\$264.9 billion	~\$264.9 billion
<i>Total outpatient/routine exposure</i>	~\$135.2 billion	~\$135.2 billion
<i>Outpatient repricing pressure</i>	-\$13.52 billion	Included
<i>UC/liquidity offset</i>	+\$1.91 billion	Included
<i>Bounded stabilization support</i>	+\$0.30 billion	Included
<i>Net provider impact</i>	-\$11.30 billion	Approximately -\$11.08 billion
<i>Net impact as share of NPR</i>	-4.3%	Approximately -4.2%
<i>Rural access guarantee</i>	Not separately enhanced	2% of NPR, capped at \$5 million per rural provider
<i>Rural implementation floor</i>	Not separately enhanced	No qualifying rural provider below -1.5% of NPR
<i>Behavioral institutional adjustment</i>	Standard modeled repricing	25% of standard modeled repricing pressure
<i>Behavioral implementation floor</i>	Not separately enhanced	No qualifying behavioral provider below -5% of NPR

Source: Author analysis of FY2024 CMS HCRIS hospital cost reports and UCC/HSE provider-impact scoring model. (67)

7.3 Base/Base Provider-Impact Result

The central Base/Base provider-impact scenario uses a 10 percent routine/outpatient repricing rate, a 25 percent uncompensated-care/liquidity offset rate, and a 1.0× bounded stabilization multiplier. This scenario is not a prediction of every provider's final equilibrium position. It is a static first-order estimate of the distributional pressure created by moving routine care toward HSE-financed transparent markets **(67)**.

The Base/Base result estimates approximately \$13.52 billion in outpatient repricing pressure, offset by approximately \$1.91 billion in uncompensated-care/liquidity relief and approximately \$0.30 billion in bounded stabilization support. The net modeled provider impact is approximately -\$11.30 billion, equal to about -4.3 percent of net patient revenue across the 886 modeled providers.

The preferred access-protection sensitivity does not eliminate provider-sector price discipline. It adds targeted rural and behavioral-health protections while preserving the core routine-care repricing mechanism. Under this specification, the modeled net provider impact improves modestly to approximately -\$11.08 billion. The purpose is not to hold the entire provider sector harmless. The purpose is to identify where revenue re-composition could threaten essential access and to address those cases with bounded, formula-based support.

7.4 Uncompensated Care and Bad-Debt Relief

UCC/HSE affects providers through both negative and positive channels. Routine outpatient repricing reduces revenue for providers currently benefiting from opaque, high-margin routine billing. At the same time, HSE deposits, catastrophic coverage, and improved household liquidity reduce uncompensated care and bad debt. Existing evidence on uncompensated care, Medicaid disproportionate-share hospital payments, and nonprofit hospital charity-care/tax-exemption policy provides the relevant institutional context for this offset channel **(49–52)**.

The master model reports a provider-facing uncompensated-care baseline of approximately \$53.0 billion in 2026, rising to \$89.5 billion by 2035, with a ten-year baseline of approximately \$698.6 billion. Avoided emergency-department overutilization, bad-debt reduction, and charity-care tax effects are modeled separately because they operate through different channels. Avoided emergency-department use reduces pressure on hospitals from non-emergency utilization. Bad-debt reduction improves collections. Charity-care tax effects operate indirectly through the reduced charity-care burden **(47–52)**.

This distinction matters because hospitals are not affected only by price compression. They are also affected by whether patients have liquid funds for routine care, whether catastrophic bills are insured, whether avoidable emergency visits decline, and whether collections improve. A system that produces transparent routine-care markets but leaves low-income households without liquidity would likely increase uncompensated care. UCC/HSE is designed to avoid that outcome by prefunding HSE deposits for low-income and chronic-condition households and maintaining universal catastrophic protection.

The provider model therefore does not score routine-care repricing in isolation. It pairs repricing pressure with UC/liquidity offsets and stabilization support. That structure is more realistic than a simple price-cut model because it captures both revenue loss from routine-market discipline and revenue improvement from reduced uncompensated exposure.

7.5 Rural Access-Capacity Payment

Rural providers require separate treatment because their constraints are structural. Low volume, fixed standby capacity, broad service areas, thin labor markets, long transfer distances, and lack of nearby substitute providers distinguish rural hospitals from ordinary outpatient competitors. In many rural markets, transparent routine-care pricing alone cannot finance essential access capacity. At the same time, exempting rural providers from all price discipline would undermine the UCC/HSE architecture.

The model resolves this tension by separating rural access-capacity support from general transition stabilization. Transition stabilization is temporary implementation-period support for providers facing acute adjustment pressure. Rural access support is treated as a durable access-capacity policy because rural constraints may persist indefinitely. The rural access payment is therefore provider-side, formula-based, capped, and separate from the household benefit design.

The preferred rural policy is a permanent rural access-capacity payment equal to 2 percent of net patient revenue, capped at \$5 million per qualifying rural provider. This payment is paired with an implementation-period floor ensuring that no qualifying rural provider falls below -1.5 percent of net patient revenue during the transition period. The permanent payment recognizes geography and standby-capacity constraints. The temporary floor protects against transition shock.

Under the rural baseline, rural / CAH / IHS providers show approximately -\$0.341 billion in net impact, equal to -3.1 percent of net patient revenue, with 10 providers below -5 percent of NPR. Under the preferred rural access package, added support of approximately \$0.218 billion improves rural net impact to approximately -\$0.123 billion, or -1.1 percent of NPR,

and eliminates modeled rural providers below the -5 percent and -10 percent NPR thresholds (67).

This result is important because it preserves the core economic logic of UCC/HSE while addressing the strongest provider-access objection. Rural providers are not exempted from all repricing. They are protected from access-threatening modeled losses. The support is capped, formula-based, provider-side, and tied to access capacity rather than to restoration of opaque routine-care billing.

Table 8. Rural Access Before and After Preferred Access Protection

<i>Rural access measure</i>	Rural baseline	Preferred rural access protection
<i>Rural / CAH / IHS net impact</i>	-\$0.341 billion	-\$0.123 billion
<i>Net impact as share of rural NPR</i>	-3.1%	-1.1%
<i>Added rural access support</i>	—	+\$0.218 billion
<i>Rural access-capacity payment</i>	None beyond base model	2% of NPR, capped at \$5 million per provider
<i>Implementation-period floor</i>	None beyond base model	No qualifying rural provider below -1.5% of NPR
<i>Rural providers below -5% of NPR</i>	10 providers	0 providers
<i>Rural providers below -10% of NPR</i>	Not retained under preferred policy	0 providers

Source: Author analysis of FY2024 CMS HCRIS hospital cost reports and UCC/HSE rural access sensitivity model. (67)

7.6 Implementation-Period Floor

The implementation-period floor is separate from the permanent rural access-capacity payment. Its purpose is transition protection. During the early years of UCC/HSE, providers must adapt to HSE-financed routine-care markets, transparent pricing, service-line redesign, chronic-care bundles, telehealth integration, and new reimbursement flows. Some providers may face acute adjustment pressure even if they are viable in the long run.

The floor prevents transition shocks from producing avoidable access loss. For qualifying rural providers, the preferred model prevents net impact from falling below -1.5 percent of

net patient revenue during the implementation period. For qualifying behavioral-health institutions, the preferred sensitivity includes a floor preventing modeled impact below -5 percent of NPR. These floors are not permanent guarantees of existing business models. They are time-limited protections against abrupt disruption **(67)**.

This distinction should remain explicit. UCC/HSE does not hold every provider harmless. It does not subsidize executive compensation, unrelated capital expansion, or preservation of opaque facility-fee billing. It supports essential access capacity while forcing providers to adapt to a more price-disciplined routine-care environment.

7.7 Rural Transformation and Transparency Conditions

Rural stabilization is not unconditional. Providers receiving access support participate in transparency, reporting, and transformation obligations. These include cash-price publication for routine HSE-eligible services, disclosure of bundle prices, HCRIS-compatible cost reporting, access monitoring, referral and transfer protocols, no-surprise and no-balance-billing rules for catastrophic episodes, reporting on rural stabilization payments, and telehealth readiness or participation in a rural virtual access network.

This conditionality prevents rural stabilization from becoming a back door to comprehensive-plan inflation. Rural providers receive support because geographic access is structurally valuable, not because opaque outpatient markups are protected. The policy exchange is straightforward: durable access-capacity support in return for transparency, reporting discipline, operational flexibility, and adaptation to HSE-financed routine-care markets.

The rural transformation track reinforces this logic. Eligible uses include financial and operational redesign, transparent price-posting infrastructure, chronic-care bundle development, telehealth integration, care-management redesign, diagnostic and laboratory access coordination, workforce cross-training, and service-line planning for low-volume inpatient units. Prohibited uses include patient deductible reduction, insurance riders, commercial rate increases, unrelated capital expansion, executive compensation, or preservation of opaque facility-fee billing.

7.8 Interpreting the Provider Model

The provider-impact estimates should be interpreted carefully. They are static, first-order estimates based on FY2024 HCRIS data. They do not predict every closure, merger, service-line change, capital-market response, bargaining adjustment, or long-run equilibrium price. They do not replace future CMS, CBO, MedPAC, state-level, or actuarial review. Their value is that they convert the provider-impact question into a reproducible scoring framework

with transparent inputs, classification correction, scenario parameters, stabilization logic, rural access protection, and sensitivity testing **(67)**.

That is the proper role of Table 7. It does not prove that no provider will be harmed. It demonstrates that UCC/HSE's provider effects are analyzable, bounded, and partly stabilizable without abandoning the core reform. The model shows that routine-care repricing produces real pressure; that uncompensated-care and liquidity offsets reduce some of that pressure; that rural providers require targeted access-capacity support; and that the preferred access package can eliminate modeled severe rural tail risk while preserving price discipline.

The answer to “what happens to hospitals?” is therefore not that hospitals are unaffected. Nor is it that hospitals are uniformly cut. The answer is that the provider sector is re-composed. High-acuity and catastrophic care remains insured. Routine outpatient revenue faces price discipline. Providers currently dependent on opaque routine-care margins must adapt. Access-sensitive providers receive bounded stabilization. Rural providers receive durable access-capacity support where geography prevents ordinary market substitution. That is the provider-side equilibrium of UCC/HSE.

8. Comparison to ACA and Medicare for All

UCC/HSE occupies a different structural category than both the ACA and Medicare for All. It is not a continuation of the ACA's premium-subsidy architecture, and it is not a single-payer federalization of comprehensive health financing. It is a universal catastrophic framework paired with routine-care liquidity and federal high-cost-claim stabilization.

The comparison is therefore not merely ideological. It is structural. The ACA subsidizes comprehensive insurance premiums. Medicare for All federalizes comprehensive financing. UCC/HSE limits the federal role to catastrophic protection, targeted HSE liquidity, and reinsurance while moving routine care toward transparent direct financing.

8.1 ACA: Open-Ended Premium-Linked Subsidies

The ACA expanded coverage by subsidizing comprehensive insurance. Premium tax credits are tied to benchmark premiums. Cost-sharing reductions alter the actuarial value of silver plans. Essential health benefit requirements preserve broad benefit design. Exchange administration determines eligibility, reconciles subsidies, and manages plan participation. This architecture protects many households from immediate coverage loss, but it leaves federal spending exposed to premium growth. CBO's federal health-insurance subsidy baseline provides the relevant current-law reference point for marketplace subsidies and related federal coverage commitments **(3)**.

The fiscal issue is not that ACA subsidies exist. The issue is that the subsidy base is comprehensive-plan inflation. When benchmark premiums rise because of provider pricing power, benefit mandates, administrative load, insurer-market dynamics, or utilization growth, federal premium subsidies rise with them. The ACA therefore subsidizes both protection and the inflationary structure embedded in comprehensive insurance.

UCC/HSE rejects that subsidy base. It preserves universal financial protection against major medical events, but it does not tie federal support to comprehensive premiums. Catastrophic credits attach to a standardized catastrophic layer. HSE deposits finance routine-care liquidity through explicit formulas. Reinsurance stabilizes high-cost claims. The federal obligation is therefore moved away from comprehensive-plan inflation and toward defined categories of support.

8.2 Medicare for All: Federalization of Comprehensive Financing

Medicare for All moves in the opposite direction. Rather than limiting federal exposure to catastrophic risk and targeted routine-care liquidity, it federalizes comprehensive financing. Private premiums, employer contributions, many out-of-pocket payments, and state-level financing are replaced by federal spending. Depending on the version, cost-sharing may be reduced or eliminated, benefits may be expanded, provider payment may be compressed, and long-term services may be incorporated into the federal program.

The major Medicare-for-All estimates differ because they make different assumptions about provider reimbursement, utilization, administrative savings, drug prices, long-term care, and benefit generosity. Urban's 2019 analysis estimated that comprehensive single-payer reform would increase federal spending by tens of trillions of dollars over ten years. RAND emphasized that results are highly sensitive to payment-rate assumptions, with total national spending depending heavily on whether provider reimbursement is reduced toward Medicare levels and how utilization responds to lower cost-sharing.

Mercatus/Blahous estimated large additional federal budget commitments over the first ten years of implementation. PERI, CRFB, and KFF analyses likewise show that Medicare-for-All financing depends heavily on assumptions about tax replacement, provider payment, administrative savings, drug pricing, and benefit design **(57–63)**.

Those estimates should not be read as interchangeable. Some measure federal spending shifts; some estimate total national health expenditure; some incorporate aggressive provider-payment reductions; others assume more limited savings. But they converge on one basic point: Medicare for All represents a historically large shift of health financing onto the federal budget.

8.3 UCC/HSE: Catastrophic Protection, HSE Liquidity, and Reinsurance

UCC/HSE takes a different path. It does not federalize comprehensive care. It federalizes the catastrophic guarantee and targeted routine-care liquidity while preserving private delivery, employer participation, state roles, and transparent routine-care markets. The federal government finances catastrophic credits, HSE deposits, and reinsurance. Routine care is not converted into a universal first-dollar federal benefit.

This difference explains the fiscal contrast. In the current UCC/HSE scoring model, gross federal outlays over 2026–2035 are approximately \$11.57 trillion, with a net federal transition cost of approximately \$763.1 billion over the same period, followed by substantial long-run savings. By contrast, Medicare-for-All estimates generally imply tens of trillions of dollars in additional federal financing over a decade, depending on benefit design and provider-payment assumptions (57–63).

Federal Budgetary Effects of Major Health Reform Frameworks

(Billions of dollars, nominal)

Framework	Source	10-Year (2026–2035)	20-Year (2036–2045)	30-Year (2046–2055)	Metric Definition
Current Law (ACA baseline)	CBO (current law projections)	13,300	—	—	Federal subsidies (non-Medicare population)
Medicare for All (Comprehensive Single-Payer)	Urban Institute [57]	34,000	—	—	Additional federal spending
	RAND Corporation [58]	32,000–34,000	—	—	Total/federal spending (scenario-dependent)
	Mercatus Center (Blahous) [59]	32,600	—	—	Additional federal spending
	Friedman [69]	13,800	—	—	Total federal cost (high-savings scenario)
	PERI (Pollin et al.) [61]	~30,000	—	—	Total national spending (net savings possible)
	CRFB [62]	25,000–35,000	—	—	Additional federal financing (range)
	KFF [63]	—	—	—	No single estimate (design comparison)
UCC + HSE (Universal Catastrophic Coverage + HSE)	Author model (CBO-style scoring workbook)	763	–7,463	–32,934	Net federal impact
		(11,570 outlays / 10,807 savings)	—	—	Gross outlays and savings (10-year)

Notes: 1. Estimates for Medicare for All reflect published analyses of comprehensive single-payer proposals and are not directly comparable due to differences in benefit design, provider payment assumptions, and treatment of long-term care.
2. “Additional federal spending” refers to increases relative to current law, not total national health expenditures.
3. UCC + HSE estimates are derived from the accompanying scoring workbook and reflect a structural reallocation of health spending, with federal exposure limited to catastrophic coverage, targeted HSA support, and reinsurance.
4. Negative values indicate net federal savings relative to current-law baseline projections.
5. All figures are presented in nominal dollars and reflect model-specific assumptions regarding utilization, pricing, and population eligibility.

Table 9: Existing Medicare-for-All estimates generally imply **\$25 trillion to \$35 trillion in additional federal financing over ten years**, with leading estimates from Urban Institute and Mercatus at roughly **\$34.0 trillion** and **\$32.6 trillion**, respectively. By contrast, the UCC/HSE scoring model used in this paper projects roughly **\$11.57 trillion in gross federal outlays** over 2026–2035 and a **net federal impact of approximately +\$763.1 billion** over the same window, followed by substantial long-run savings.

The relevant distinction is not that UCC/HSE spends little. It does not. It creates large federal outlays for catastrophic credits and HSE deposits. The distinction is that UCC/HSE uses those outlays to replace premium-linked comprehensive subsidies with bounded catastrophic and routine-liquidity commitments. Medicare for All shifts comprehensive financing to the federal government. The ACA subsidizes comprehensive premiums through regulated private plans. UCC/HSE separates catastrophic risk from routine-care financing and limits federal exposure to defined policy functions.

8.4 Structural Comparison

Table 10. Structural Comparison: ACA, Medicare for All, and UCC/HSE

<i>Feature</i>	ACA	Medicare for All	UCC/HSE
<i>Core financing model</i>	Subsidized comprehensive private insurance	Federalized comprehensive financing	Universal catastrophic coverage plus HSE-financed routine care
<i>Federal exposure</i>	Linked to benchmark premiums and subsidies	Broad federal responsibility for most medical spending	Catastrophic credits, HSE deposits, and reinsurance
<i>Routine care</i>	Embedded in comprehensive insurance	Generally federally financed as part of comprehensive coverage	Financed through HSE accounts, direct payment, employer deposits, and targeted support
<i>Catastrophic care</i>	Covered through comprehensive plans	Federally financed	Universally insured through standardized catastrophic layer
<i>Employer role</i>	Preserved through tax exclusion and group plans	Largely displaced or heavily reduced	Recomposed toward wages, HSE deposits, catastrophic facilitation, and limited supplemental benefits
<i>Medicaid</i>	Expanded for low-income adults; retained for traditional populations	Often absorbed or restructured depending on proposal	Refocused on LTSS, ABD, medically complex children, HCBS, and CHIP continuity
<i>Medicare</i>	Preserved	Becomes model/platform for broader federalization	Preserved unchanged
<i>Main fiscal risk</i>	Premium-linked subsidy growth	Federalization of comprehensive spending	Transition cost and calibration of catastrophic/HSE parameters
<i>Main cost-control mechanism</i>	Subsidy regulation, plan competition, risk adjustment	Provider-payment compression, administrative savings, drug negotiation	Catastrophic thresholding, formula-based deposits, reinsurance, employer-exclusion reform, routine-care price discipline

Table 10 highlights why UCC/HSE should not be evaluated as either a narrower ACA plan or a lighter version of Medicare for All. It has a different fiscal architecture. The ACA's problem is that federal support rises with comprehensive premium inflation **(3)**. Medicare for All's challenge is that comprehensive financing moves onto the federal budget and depends

heavily on payment compression, administrative savings, and new federal financing to remain affordable **(57–63)**. UCC/HSE instead uses universal catastrophic protection and HSE liquidity to preserve financial security while moving routine care out of comprehensive premium financing.

8.5 Interpretation

The policy choice is not simply between markets and government. The relevant question is which parts of health financing require public pooling and which parts require transparent routine-care markets. Catastrophic medical events justify broad pooling. Routine and predictable services require liquidity, price visibility, and support for vulnerable households, but they do not necessarily require comprehensive insurance intermediation.

The ACA attempted to expand coverage within the existing comprehensive-insurance model. Medicare for All would replace that model with federal comprehensive financing. UCC/HSE proposes a third structure: universal catastrophic protection without federalizing all care, routine-care liquidity without comprehensive premium inflation, and fiscal guardrails without abandoning vulnerable populations.

That is the central comparative claim. UCC/HSE is not less ambitious than the ACA or Medicare for All; it is ambitious in a different direction. It attempts to redesign the financing boundary itself.

9. Implementation Pathway and Guardrails

UCC/HSE is designed as a phased replacement architecture, not an overnight conversion of the health-care system. The reform changes federal subsidies, insurer product design, employer compensation structures, household routine-care financing, Medicaid acute-care pathways, state administration, and provider payment incentives. A credible implementation pathway therefore requires sequencing, no-lapse coverage rules, fiscal caps, benefit-design enforcement, and program-integrity controls.

The transition is structured over a three-to-five-year period. This sequencing allows households, employers, insurers, states, HSE custodians, and providers to adjust while preventing indefinite coexistence of two incompatible subsidy systems. The proposed transition schedule is straightforward: Year 1 makes UCC catastrophic plans available and builds the statutory and administrative foundation; Year 2 closes ACA exchanges to new enrollment; Year 3 phases out ACA subsidies and phases in UCC credits; Years 4–5 complete the Medicaid expansion adult and employer-market transition.

9.1 Year 1: Legislative Authorization, Infrastructure, and Initial UCC Availability

Year 1 establishes the statutory and administrative foundation. Congress authorizes the UCC catastrophic-insurance category, creates the national reinsurance pool, establishes the universal catastrophic credit, authorizes Treasury-administered HSE accounts, sets the ACA subsidy phaseout schedule, and gives states a pathway to consolidate existing Section 1332 waiver reinsurance arrangements into the national model. The implementation framework specifies that CMS and Treasury develop standardized catastrophic-plan templates, actuarial guidance, HSE distribution and verification systems, and reinsurance disbursement protocols during the first year **(4)**.

This first year also launches the account infrastructure. Treasury administers HSE accounts through a national custodial platform, with automatic account issuance at birth, naturalization, or eligibility determination and optional transfer to private custodians. Federal deposits are delivered through standardized ACH operations and integrated into annual IRS reporting. This structure reduces fragmentation, supports portability, and creates a national platform for routine-care liquidity.

Year 1 also begins program-integrity development. Eligibility verification should use IRS, SSA, CMS, and other relevant federal data systems. HSE custodians must be certified before receiving federal deposits. Insurers must submit UCC-compliant catastrophic-plan filings, solvency documentation, claims-reporting protocols, and reinsurance audit procedures. Providers receiving stabilization or rural access-capacity payments must comply with reporting requirements, price-disclosure obligations, and HCRIS-compatible financial review **(53–56,67)**.

9.2 Year 2: UCC Plans Available and ACA Exchanges Closed to New Enrollment

In Year 2, certified UCC catastrophic plans become broadly available through simplified federal, state, private, and employer-facing enrollment channels. Existing ACA exchanges remain operational only for transition administration, but they close to new enrollment. New enrollment shifts toward UCC catastrophic plans paired with HSE accounts.

This stage is important because it prevents the ACA and UCC/HSE from becoming permanent parallel systems. Households already enrolled in ACA coverage remain protected during the transition, but new federal support begins moving toward the catastrophic-and-HSE architecture. Insurers price standardized catastrophic plans under national benefit rules and federal reinsurance parameters. Employers begin converting portions of health-benefit compensation into HSE contributions, catastrophic-plan facilitation, or wages.

States also begin converting existing Section 1332 waiver reinsurance structures into UCC-compatible reinsurance mechanisms. State experience with reinsurance validates a narrower actuarial claim: high-cost-claim stabilization can reduce premium volatility and support risk-pool stability. The national UCC/HSE model builds on that mechanism but does not treat state reinsurance waivers as proof of the entire reform architecture **(4)**.

9.3 Year 3: ACA Subsidies Phase Out and UCC Credits Phase In

Year 3 is the central fiscal conversion year. ACA premium tax credits, cost-sharing reductions, and exchange-based subsidy mechanisms phase out for the replacement population. UCC catastrophic credits and HSE deposits phase in as the primary federal financing mechanisms. This is the moment when federal support shifts from comprehensive benchmark premiums to a defined catastrophic product and formula-based routine-care liquidity. CBO's current-law health-insurance subsidy baseline provides the comparison point for this conversion **(3)**.

Continuity protection is essential during this phase. No eligible person with qualifying ACA exchange coverage, employer-sponsored insurance, or Medicaid expansion coverage at enactment should experience a lapse in catastrophic protection during the first transition years, provided the individual complies with enrollment procedures. The legislative outline in the master paper includes a no-lapse guarantee and CHIP/pediatric continuity provisions to prevent administrative gaps during exchange wind-down.

Children require additional continuity rules. Children enrolled in Medicaid or CHIP at enactment should retain coverage continuity during transition, with pediatric HSE deposits coordinated so that routine-care financing begins before Medicaid/CHIP eligibility ends. The operational principle is simple: catastrophic enrollment and HSE account activation must precede the end of any legacy coverage pathway.

9.4 Years 4–5: Medicaid Expansion Adult and Employer Transition

Years 4 and 5 complete the transition. Medicaid remains responsible for long-term services and supports, Aged/Blind/Disabled populations, medically complex children, home- and community-based care, and CHIP continuity. Non-disabled adult acute-care populations transition into UCC catastrophic coverage and HSE-financed routine care.

Employer transition also completes during this period. UCC/HSE does not abolish employer participation. It changes the tax treatment and structure of that participation. Employers increasingly support workers through HSE contributions, wage adjustments, UCC catastrophic-plan facilitation, and limited supplemental benefits rather than through tax-preferred comprehensive premiums. The employer-market transition is grounded in the

fiscal and labor-market significance of the employer-sponsored insurance tax exclusion and employer health-benefit compensation **(21,31–33,64,65)**.

The collective-bargaining rule protects existing negotiated agreements. Amounts excludable under Section 106 of the Internal Revenue Code remain excludable for the duration of collective-bargaining agreements in force at enactment; upon expiration, the general phase-down schedule applies. This avoids abrupt disruption in unionized sectors while preserving the long-run transition to a wage-transparent and portable financing structure.

9.5 Fiscal Guardrails

UCC/HSE requires statutory fiscal discipline. The combined federal cost of catastrophic credits, HSE deposits, reinsurance, rural access-capacity payments, and transition stabilization must be constrained by a benchmark tied to projected current-law ACA subsidy and cost-sharing obligations. If outlays exceed the statutory cap beyond a defined tolerance band, automatic correction mechanisms should apply in the following plan year.

The legislative outline establishes a federal outlay cap, automatic adjustment mechanism, and annual budgetary/program-performance reporting requirement. If total outlays exceed the cap, automatic adjustments may apply to catastrophic deductible indexing and/or HSE deposit levels, subject to protected statutory floors. Those floors should protect low-income HSE deposits, pediatric deposits, chronic-condition supplements, rural household access supplements, behavioral-health continuity support, and rural provider access-capacity payments unless Congress affirmatively amends them.

Treasury and HHS should issue annual CBO-style and MedPAC-style reports to Congress detailing federal outlays, savings relative to the ACA baseline, distributional effects, provider-impact stabilization, rural access effects, behavioral-health institutional effects, and operation of automatic correction mechanisms. The purpose is to prevent UCC/HSE from becoming a new open-ended entitlement disguised as reform.

9.6 Benefit-Creep Guardrails

Benefit creep is the central institutional threat to UCC/HSE. A universal catastrophic system can maintain cost discipline only if the boundary between catastrophic insurance and supplemental coverage remains enforceable. Without clear statutory limits, insurers, employers, and supplemental vendors would rebuild comprehensive insurance through riders, fixed-indemnity products, deductible-offset products, or employer-funded wraparound benefits.

The supplemental-coverage framework permits only narrow supplemental products: elective or ancillary services such as dental, vision, and short-term disability; defined service bundles such as maternity or musculoskeletal episodes; pre-deductible primary-care arrangements such as direct primary care or integrated physician models; and voluntary catastrophic-deductible financing mechanisms. It prohibits routine care on a claim-by-claim basis, coverage of services below the catastrophic deductible, generalized wraparound major-medical coverage, and benefits that replicate ACA gold or platinum actuarial values.

The actuarial boundary is explicit: supplemental plans must maintain actuarial value at or below 15 percent, may not raise pre-deductible coverage above federal thresholds, and must remain strictly outside the catastrophic structure. Supplemental plans must disclose premiums, claims ratios, actuarial value, benefit limits, and the percentage of the catastrophic deductible they offset. Employer contributions are restricted to HSE accounts; supplemental products must be purchased with after-tax dollars; and employers may not condition employment on supplemental coverage.

9.7 Program Integrity

UCC/HSE is simpler than the ACA, but it still requires strict policing. HSE withdrawals should be limited to qualified health expenses and subject to reporting, audit, and penalties for misuse. Reinsurance claims should be audited to prevent upcoding, claim splitting, service reclassification, and improper treatment of routine claims as catastrophic exposure. Insurers should face penalties for plan designs that evade supplemental-coverage limits. Providers receiving rural or stabilization payments should meet price-disclosure, access-monitoring, and reporting obligations.

Program integrity should also apply to eligibility. The system relies on verified eligibility, income data, household composition, citizenship or eligible residency status, Medicare exclusion, Medicaid transition status, and chronic-condition certification. Cross-agency verification is therefore not optional. It is the administrative foundation for a fiscally bounded system.

9.8 Countercyclical Liquidity Stabilizer

A catastrophic-first system must preserve routine-care liquidity during economic downturns. Recessions reduce household contributions and employer deposits precisely when cash-on-hand becomes more important. UCC/HSE therefore includes an automatic HSE stabilizer triggered when the national unemployment rate rises 1.0 percentage point above its trailing twelve-month average or when the economy experiences two consecutive quarters of negative real GDP growth.

When activated, federal HSE seeding increases by \$300 per adult and \$150 per dependent child, with proportional increases for chronic-condition bundles. The enhanced seeding phases out when unemployment returns within 0.5 percentage points of its pre-trigger average or when real GDP growth exceeds 1 percent for two consecutive quarters. Total recession-triggered supplementation is capped at 0.05 percent of GDP in any fiscal year.

This stabilizer preserves household liquidity without reintroducing comprehensive insurance. It is rules-based, temporary, capped, and tied to macroeconomic conditions rather than discretionary emergency appropriations.

10. Risks and Limitations

UCC/HSE addresses structural failures in American health financing, but it does not eliminate uncertainty. A reform that replaces a mature subsidy architecture, changes employer incentives, moves routine care toward direct financing, creates a national catastrophic layer, and restructures Medicaid acute-care pathways will face behavioral, institutional, fiscal, and administrative risks. These risks do not defeat the model, but they require ongoing calibration, empirical monitoring, and policy discipline.

10.1 Behavioral Adjustment Risk

The shift from comprehensive first-dollar insurance toward HSE-financed routine care requires behavioral adjustment. Households accustomed to copays, managed networks, and opaque insurer-negotiated prices may initially underuse routine services, overuse HSE funds, delay price comparison, or remain uncertain about which services belong in the routine-care layer. Providers may also be slow to publish usable cash prices or organize routine-care bundles.

The principal policy concern is underutilization among low-income households, chronic-condition patients, children, and behavioral-health patients. Evidence on high-deductible plans, HSAs, preventive-service use, medication adherence, and chronic-care management shows that poorly designed cost-sharing can reduce necessary care or fail to generate durable consumer cost-consciousness when liquidity and clinical safeguards are inadequate **(36–40)**. UCC/HSE mitigates this risk through annual prefunding, pediatric deposits, chronic-condition supplements, income-based early-year support, and the ER non-backflow guardrail. The framework emphasizes that low-income and chronic-condition deposits are made at the start of the benefit year, reducing deferred care, medical debt accumulation, and avoidable emergency-department substitution.

The risk remains empirical. Household behavior must be measured after implementation through HSE utilization data, preventive-care adherence, chronic-care monitoring, ER utilization, and delayed-care indicators.

10.2 Provider Consolidation and Local Market Power

UCC/HSE strengthens price discipline in routine-care markets, but it cannot independently dismantle entrenched provider monopolies. In regions dominated by consolidated hospital systems, routine cash prices may remain elevated even when care is financed through HSE accounts. Insurers may still need dominant hospital systems for catastrophic networks. Federal reinsurance may still absorb high claims generated in high-price regions.

Competition policy therefore remains an essential complement. Antitrust enforcement, restrictions on anti-competitive contracting, site-neutral payment reform, certificate-of-need reform, scope-of-practice modernization, telehealth liberalization, and transparent facility-fee rules all affect whether UCC/HSE produces robust routine-care competition. The reform improves the environment in which competition can occur; it does not guarantee competition in every local market. Evidence on commercial price variation, provider payment differences, price transparency, and network oversight supports the conclusion that local market structure can limit the effectiveness of transparency alone **(5–8,15,20,34,35,53–56)**.

10.3 Chronic-Condition Calibration Risk

Chronic-condition funding is one of the most sensitive design areas. If HSE deposits and chronic-condition supplements fall below real-world disease-management costs, patients with diabetes, congestive heart failure, COPD, chronic kidney disease, severe mental illness, addiction-management needs, or other recurring conditions may face liquidity pressure beneath the deductible. If supplements are too broad or too generous, the system may recreate comprehensive insurance through disease-specific funding channels.

The model identifies this as an ongoing calibration problem. Chronic-condition support depends on expenditure-triggered funding pathways, statutory minimum deposits indexed to observed disease-management costs, and periodic recalibration using CMS, MEPS, FAIR Health, and claims-level data **(20,44,46,66)**. Evidence on high-deductible plan effects for chronic illness also supports the need for careful chronic-condition safeguards rather than simple deductible exposure **(36–40)**. These mechanisms reduce systematic underfunding risk, but they do not eliminate regional and clinical variation.

10.4 Macroeconomic Shock Risk

Economic downturns create a distinct risk. Household HSE contributions and employer deposits may weaken during recessions, while routine-care needs and financial stress may

rise. Without a countercyclical stabilizer, a cash-pay routine-care environment could become pro-cyclical even if catastrophic protection remains intact.

UCC/HSE addresses this risk through automatic HSE augmentation tied to unemployment or negative GDP growth, with a 0.05 percent of GDP annual cap. This reduces pro-cyclical liquidity strain while preserving fiscal discipline. Severe or prolonged recessions may still require temporary federal support beyond the automatic stabilizer, but no catastrophic-first framework can fully neutralize macroeconomic shocks. The relevant design question is whether the architecture contains an explicit stabilizer; UCC/HSE does.

10.5 Rural Provider Vulnerability

Rural providers face a separate structural risk. Many rural hospitals rely on cross-subsidies from elevated commercial outpatient prices to maintain standby capacity despite low patient volume, high fixed costs, and long travel distances. By shifting routine care toward transparent pricing and reducing the role of comprehensive insurance in outpatient payment, UCC/HSE may compress implicit cross-subsidies that currently support rural facilities.

The framework addresses this through rural access-capacity payments, a temporary rural implementation-period floor, HCRIS-based monitoring, rural transformation assistance, and access review for closures or major service-line reductions. These mechanisms function collectively as the rural stability framework (67). The model distinguishes permanent rural access support from temporary transition protection and emphasizes that rural providers are not exempted from repricing, but access-threatening modeled losses are eliminated under the preferred policy.

The limitation remains real. Rural stabilization must protect access capacity without preserving opaque outpatient markups indefinitely.

10.6 Supplemental Benefit Creep and Insurance Gaming

Supplemental benefit creep is a long-run structural vulnerability. Even with a uniform catastrophic design, insurers, employers, unions, and supplemental vendors may attempt to reconstruct de facto comprehensive coverage through riders, fixed-indemnity add-ons, deductible-offset products, or outpatient bundles that function as first-dollar coverage.

If benefit creep succeeds, UCC/HSE would gradually lose its central fiscal logic. Routine care would migrate back into insurance-like products, actuarial-value expectations would rise, federal catastrophic credits could indirectly support quasi-comprehensive arrangements, and price opacity would return. The model therefore requires strict statutory definitions, actuarial-value limits, standardized disclosures, federal auditing, and

employer-side guardrails. Existing transparency rules and hospital-price-transparency experience show why disclosure and enforcement mechanisms must be explicit rather than assumed **(16,17,53–56)**.

This risk is manageable only if future policymakers resist pressure to loosen the boundary between catastrophic insurance and supplemental coverage.

10.7 Transition and Administrative Risk

Implementation risk is substantial. Federal and state agencies must coordinate eligibility systems, HSE account issuance, IRS income verification, insurer certification, exchange wind-down, Medicaid redetermination, employer-exclusion transition, reinsurance audits, supplemental-product oversight, program-integrity enforcement, and provider stabilization. Misalignment could produce confusion, duplicative enrollment, payment delays, temporary coverage uncertainty, or administrative burden during the transition.

The phased implementation schedule reduces this risk but does not remove it.

Administrative simplicity is an objective of the final system; it is not guaranteed during conversion. Treasury, HHS, CMS, IRS, state insurance departments, HSE custodians, insurers, employers, and providers must operate from a common rulebook.

10.8 Empirical Uncertainty

The UCC/HSE model uses MEPS-based expenditure reconstruction, CMS spending projections, HCRIS provider data, CBO/JCT subsidy baselines, and internal CBO-style scoring assumptions. These sources make the model transparent and reproducible, but they do not eliminate uncertainty. MEPS is survey-based and not a full claims microsimulation. HCRIS is provider-reported cost-report data and does not predict dynamic market behavior. Long-run savings depend on assumptions about routine-care repricing, employer-exclusion phase-down, administrative simplification, provider adaptation, and household behavior **(1,3,21,44,46,66,67)**.

The framework correctly frames these uncertainties as characteristic of any large-scale replacement of a legacy insurance architecture. Implementation requires empirical evaluation of reinsurance calibration, chronic-condition benchmarking, consumer response, cash-price trajectories, substitution effects, and administrative performance.

The appropriate response is not to overstate precision. UCC/HSE should be treated as a reproducible first-order scoring framework requiring formal CBO, CMS, MedPAC, actuarial, and state-level review before enactment. Its credibility rests on explicit assumptions, transparent formulas, and adjustable guardrails—not on the claim that long-run projections are certain.

10.9 Interpretation

The risks are substantial but identifiable. Behavioral adjustment, provider consolidation, chronic-condition calibration, macroeconomic shocks, rural vulnerability, supplemental benefit creep, transition execution, and empirical uncertainty all require active governance. Acknowledging these risks strengthens the proposal. It shows that UCC/HSE is not a slogan or a single lever, but a policy architecture requiring discipline after enactment as well as during design.

The central limitation is also the central test: UCC/HSE works only if policymakers maintain the distinction between catastrophic insurance and routine-care financing. If that boundary erodes, the system drifts back toward comprehensive-plan inflation. If the boundary is preserved, the framework offers a plausible path toward universal catastrophic protection, routine-care price discipline, and bounded federal exposure.

Conclusion

The central failure of American health financing is structural. The United States has built a system that uses comprehensive insurance to finance both catastrophic risk and routine care. That design obscures prices, inflates administrative costs, weakens consumer discipline, rewards benefit expansion, and exposes federal taxpayers to premium-linked subsidy growth.

The ACA improved coverage but did not resolve this deeper problem. It stabilized parts of the individual market through subsidies, mandates, exchanges, and cost-sharing rules, but it preserved the basic logic of comprehensive insurance. As long as federal support is tied to premiums that reflect expansive benefit design, provider pricing power, and administrative complexity, federal health spending will remain vulnerable to inflationary feedback **(3,5–14,19,34,66)**.

UCC/HSE offers a different architecture. It restores insurance to its proper role: protection against large, unpredictable losses. It moves routine and predictable care into transparent, HSE-financed markets. It uses federal reinsurance to stabilize the catastrophic layer. It preserves Medicare. It refocuses Medicaid on long-term care, disability, medically complex children, and CHIP. It supports chronic-condition and low-income households through financing beneath the deductible rather than by recreating comprehensive insurance.

The model is not without risk. Provider consolidation may limit price competition in some markets. Chronic-condition funding must be calibrated carefully. Rural providers require targeted stabilization. Supplemental products must be regulated to prevent benefit creep.

Transition from the ACA architecture would require administrative discipline and political durability.

Those risks are real, but they are manageable within the structure of the proposal. The greater risk is continuing the current system: open-ended subsidies layered onto comprehensive plans, with weak price discovery and rising federal exposure. UCC/HSE does not claim that consumer shopping alone can solve American health care. Its fiscal logic comes from the interaction of catastrophic risk pooling, fixed HSE formulas, benchmark-linked pricing, employer-exclusion reform, administrative simplification, routine-care repricing, and provider-sector re-composition.

The purpose of health insurance should be clear: protect households from financial ruin. The purpose of routine-care financing should be equally clear: preserve access while allowing prices to become visible, contestable, and disciplined. UCC/HSE separates these functions and builds federal policy around that distinction.

The result is a framework that provides universal catastrophic protection without federalizing all care, supports vulnerable populations without perpetuating comprehensive-plan inflation, and restores a measure of market discipline without abandoning public responsibility. In doing so, UCC/HSE offers a serious alternative to both the ACA's subsidy architecture and single-payer federalization: a bounded, transparent, and institutionally durable model for American health-care reform.

Supplementary:

Github link to full 261-page white paper and reproducible R scripts for data analysis of HCRIS 2024 data and HC-251 2023 data.

<https://github.com/Galtman-droid/Universal-Catastrophic-Coverage-with-Health-Savings-Empowerment-A-Structural-Replacement-for-the-ACA>

SSRN DOI link

https://papers.ssrn.com/sol3/papers.cfm?abstract_id=6685658

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